Growing Taller, Growing Heavier: The Effects of Progresa on Adolescent Health in Urban Mexico

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Abstract

This paper estimates the causal effects of Mexico's Progresa conditional cash transfer program on children's health in urban areas, extending evidence beyond the well-known rural randomized evaluation. I exploit plausibly exogenous variation in the timing of program rollout across urban localities and children's age at first exposure, using a differences-in-differences design with repeated cross-sectional data. Focusing on cohorts exposed during early adolescence, I estimate the effects of receiving one additional year of treatment before age fourteen. I find that Progresa significantly increases boys' height and weight, with boys gaining approximately 1.9 centimeters per additional year of exposure, while effects on girls' height are smaller and imprecise. At the same time, the program increases body mass index and the prevalence of overweight and obesity, particularly among girls and children from low socioeconomic status households. These results highlight both the potential for income transfers to affect human capital accumulation beyond early childhood and important trade-offs in urban settings.

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1 Introduction

Conditional cash transfer (CCT) programs have become one of the most widely used policy instruments to reduce poverty and promote human capital accumulation in low- and middle-income countries. Among them, Mexico's Progresa program (also known as Oportunidades and Prospera) stands as a benchmark intervention, largely due to its pioneering randomized controlled trial (RCT) conducted in rural areas in the late 1990s. Evidence from this evaluation shows that Progresa improved children's health, nutrition, and schooling outcomes, particularly in early childhood, and has shaped the global expansion of CCTs (Fernald et al., 2008b; Parker and Todd, 2017). Afterwards, Progresa was rapidly scaled up nationwide, including to urban areas, under the implicit assumption that its impacts would generalize beyond the original rural experimental setting.

Despite the central role of Progresa in the development economics literature, causal evidence on its impacts in urban settings remains limited (Gertler, 2004; Behrman and Hoddinott, 2005). This gap is consequential. Urban households differ markedly from rural ones in labor markets, food environments, schooling opportunities, and access to services, all of which may alter the channels through which income transfers affect human capital formation (Popkin, 2001). Moreover, while much of the existing literature emphasizes the importance of early-life interventions—particularly during the first 1,000 days—less is known about whether income transfers can influence physical development later in childhood or adolescence, a period characterized by rapid growth and potentially meaningful scope for catch-up (Hoddinott et al., 2008; Prentice et al., 2013; Black et al., 2013).

This paper provides novel causal estimates on the effects of Progresa on children's health in urban Mexico. I study the program's impacts on anthropometric outcomes —height, weight, and body mass index (BMI)—among cohorts exposed to Progresa during early adolescence. I exploit plausibly exogenous variation in the timing of Progresa's rollout across urban localities between 2001 and 2005, combined with differences in children's age at first exposure to the program. Using repeated cross-sectional health survey data and a differences-in-differences design with cohort and locality fixed effects, I estimate the effects of receiving one additional year of Progresa before age fourteen.

The focus on adolescence is motivated by both biological and economic considerations. While linear growth is most sensitive to nutrition in early childhood, a growing body of evidence suggests that height deficits can continue to accumulate—and potentially be mitigated—beyond early childhood, particularly among disadvantaged populations (Leroy et al., 2014; Georgiadis and Penny, 2017). From a policy perspective, adolescence coincides with eligibility for Progresa's education transfers and represents a critical stage for human capital accumulation through both physical development and schooling. Understanding whether income transfers affect health during this stage is therefore central to evaluating the full lifecycle impacts of CCTs.

The data in this paper comes from two primary sources. First, I use administrative records on Progresa beneficiaries' enrollment by locality between 1999 and 2012. Second, I use the National Health and Nutrition Survey (ENSA 2000; ENSANUT 2006) to construct a repeated cross-section database by birth cohort and locality of residence, which I link to Progresa's enrollment data. My sample includes individuals born between 1983 and 1989 who were eligible to receive the education cash transfer between 2000 and 2006.[1] Following previous studies on the effectiveness of nutrition interventions, and given the biological differences observed in the children's growth trajectories, I set 14 years old as the maximum age for an individual to be effectively exposed to Progresa. This way, my treatment corresponds to the years an individual was exposed to the intervention before age fourteen.

Using before-and-after treatment data, my DiD research design models the change in children's health trajectory—as they age—that can be causally attributed to the intervention. To do this, I focus the analysis on urban localities that were not treated in 2000 (before, only 10% were treated), and I estimate Progresa's intent-to-treat (ITT) effects for children living in the intervened localities. Then, using the program's observed take-up rate in 2006, I calculate the local average treatment effect (LATE) for households

[1]Unfortunately, the ENSA 2000 only has data on health biomarkers for children over ten years old, which restricts the implementation of a differences-in-differences strategy for younger cohorts.

in the lowest socioeconomic status (SES) tercile and compare my results with those from the RCT sample.

My identification strategy relies on two main assumptions. First, the parallel trend (PT) in average children's health outcomes across localities' treatment adoption groups in staggered settings. Unfortunately, before 2000, Mexico had no other disaggregated data on children's health metrics. Instead, I test this assumption using three different proxies for children's health outcomes: locality fertility rates (1990-2012) and infant and neonatal mortality rates (1998-2012). I conduct an event study analysis (Callaway and Sant'Anna, 2021), and I do not find any pre-trends in fertility rates. Moreover, consistent with Barham (2011), I find neither significant effects of Progresa on infant mortality in urban areas nor on neonatal mortality rates.

A second assumption refers to no anticipatory effects. Given my period of interest, my design exploits the program rollout resulting from a major political change in Mexico, which was unlikely to have been anticipated. Still, if the treatment time is correlated with a locality's characteristics, my estimates would be biased if these characteristics also affect my main outcomes. Using multiple administrative data sources, I show that a locality's roll-out year is unrelated to geographic, demographic, and socioeconomic variables (this is not true for rural areas).[2]

I find that receiving Progresa before age fourteen significantly increases height and weight among boys in urban areas. Boys gain approximately 1.9 centimeters in height per additional year of exposure, with larger effects among children from low socioeconomic status households. In contrast, effects on girls' height are smaller and imprecise, consistent with earlier completion of their growth trajectory. At the same time, the program increases BMI and the prevalence of overweight and obesity, particularly among girls and low-income children. These patterns suggest that while income transfers can promote linear growth in urban settings, they may also generate unintended changes in body composition during adolescence.

[2]From government documents, it is unclear the selection criteria for new entering localities– other than emphasizing marginality and adjacency to localities already enrolled (Progresa, 2000).

This paper contributes to the literature in three main ways. First, to the best of my knowledge, it provides the first causal evidence of Progresa's impacts on children's anthropometric outcomes in urban areas, extending evidence beyond the original rural randomized evaluation (Gertler, 2004; Fernald et al., 2008b, 2009; Parker and Todd, 2017). Second, it shows that income transfers can affect physical development during adolescence, highlighting a channel through which conditional cash transfers may influence human capital accumulation beyond early childhood (Hoddinott et al., 2008; Leroy et al., 2014; Georgiadis and Penny, 2017). Third, it documents important trade-offs in urban contexts, where increased purchasing power may translate into both greater height and higher risks of overweight and obesity, a concern that has become increasingly salient as low- and middle-income countries experience rapid urbanization and a nutritional transition (Popkin et al., 2020; Vilar-Compte et al., 2021). Together, these findings inform the design and evaluation of cash transfer programs in increasingly urbanized low- and middle-income countries.

The remainder of the paper proceeds as follows. Section 2 provides background on Progresa and its expansion to urban areas. Section 3 describes the data and sample construction. Section 4 outlines the identification strategy. Section 5 presents the main results, including heterogeneity by sex and socioeconomic status. Section 6 discusses the findings in light of the existing literature and their implications for policy design, and section 7 finishes with concluding remarks and limitations of the study.

2 Background

In 1997, following a major economic crisis, the Mexican government launched an innovative strategy to alleviate poverty: Progresa (Schooling, Health and Nutrition Program). It began as a pilot randomized controlled trial (RCT) in rural Mexico. The RCT selected 506 rural localities from seven states to participate in the program. Localities were randomly assigned to treatment and control groups, and eligible households in the treatment group received the benefits 18 months earlier than those in the control group (from early 1998 to late 1999).

After the RCT concluded, Progresa was expanded to eligible families in highly impoverished rural and semi-urban municipalities in Mexico (Parker and Todd, 2017; Skoufias, 2005).

Later, in 2000, Progresa rapidly escalated to the rest of the country, including urban cities. This expansion followed a significant change in Mexico's political environment, when the incumbent party lost the presidential election for the first time in over 70 years. The program continued to function and grow over the years, and by 2016, it covered almost one-fourth of the Mexican population. Still, the intensive urban household enrollment peaked between 2001 and 2005, as new localities were incorporated each year (Figure 1). During this period, the program roll-out became less geographically targeted (by marginality classification), and its implementation differed considerably from the former rural-established program.

Initially, eligible families were notified about Progresa after a socioeconomic screening was conducted for all households. After this initial screening, eligible families received a home visit to verify their socioeconomic status. If accepted into Progresa, they remained beneficiaries for the next three years as long as they complied with their co-responsibilities. However, this census became unfeasible in urban localities. Instead, interested families in urban areas needed to attend the register office and complete the screening questionnaire to verify their eligibility. This entrance barrier led to self-selection and low program take-up, as not everyone was aware of their existence (Parker et al., 2005).

Progresa is widely known for its cash transfer (CT) conditional on school attendance. However, its multifactor design enclosed multiple benefits (Levy, 2006). These included a food CT per person, nutritional supplements, and healthcare access for all household members, conditional on the *beneficiary coresponsibilities*. As part of the conditionality, all beneficiary members were required to attend their periodic healthcare check-ups (based on their age), and one member per household — usually the mother — needed to participate in the health and nutrition workshops offered at their public clinics (every 2 or 3 months). In addition, beneficiary families with children between 3rd and 12th grade of school received the conditional education CT.[3] While the food cash aid was fixed for all beneficiary households, the education grant's amount varied by children's school grade and sex, aiming to compensate

[3]Initially, Progresa only covered up to the 9th grade. In 2001, they extended it to the 12th grade, and in 2012, they incorporated the 1st and 2nd grades for educational benefits.

for the opportunity cost of staying in school, with a maximum limit per family. The amounts were adjusted each year, and all monetary transfers were made directly to the female head of the household.

3 Data

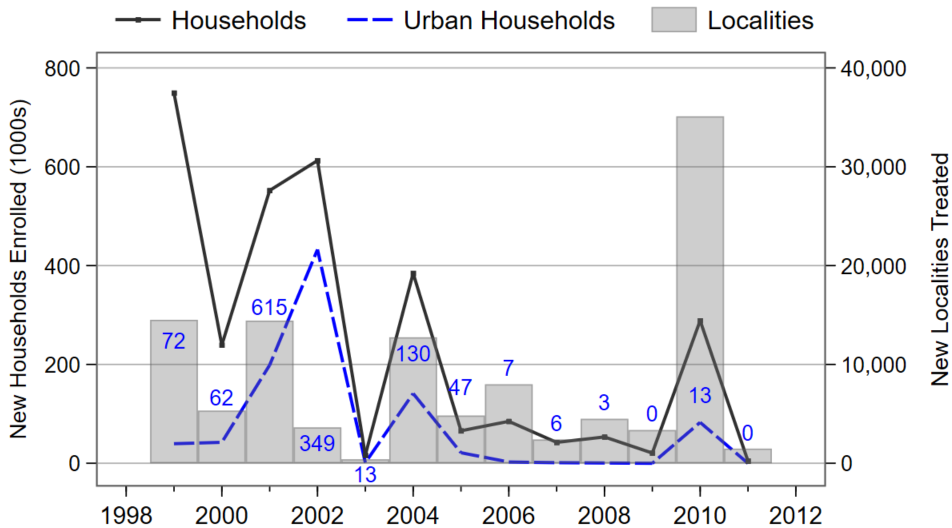
My data comes from two main sources at the locality level. First, I use administrative records on Progresa beneficiaries' enrollment between 1999 and 2012. These include the total number of families enrolled in the program by residence locality at the end of each fiscal year. A locality –the smallest geographic unit in Mexico– served as the target level to scale up Progresa.[4] I identify the year when a locality enters the intervention using the first time I observe at least one beneficiary family in the data. Following the definition of urban settlements, my sample of interest includes localities with more than 5,000 inhabitants during my study period. Then, I match this with other available sources on geographic, demographic, and socioeconomic characteristics to create a locality panel data between 1995 and 2010 ($N = 1{,}166$).[5]

Figure 1 shows the aggregate number of new households enrolled in Progresa and the total number of localities newly incorporated between 1999 and 2011. It also includes new household enrollment in urban areas (dotted line) and the number of new urban localities incorporated each year. As previously mentioned, the largest expansion in rural areas peaked in 1999 and 2010. However, the largest expansion for urban areas occurred between 2001 and 2004. Over half of the newly enrolled households in the program during this period were from urban areas. The only exception is 2003, the year of the midterm elections, when new enrollment was temporarily suspended.

[4]Mexico is composed of 32 autonomous states, divided into municipalities, and these into localities. Localities have changed throughout the years, but Mexican public records allow me to identify movements and changes in the territorial division using their 9-digit id (INEGI, 2022).

[5]These include cartographic data and Population Censuses from the National Institute of Statistics and Geography (INEGI); marginality indexes from the Population National Counsel (CONAPO); Health Resources data from the Ministry of Health (SS).

Figure 1: Progresa New Enrollment by Locality and Households, 1999-2011



Notes: Bars represent all new localities treated each year (right axis), where the number in each bar corresponds to the new urban localities treated. Source: Progresa Administrative Records.

Second, I use the National Health and Nutrition Survey (ENSANUT) and the National Health (ENSA, for its acronym in Spanish) to construct a repeated cross-sectional data set representative of Mexican children from rural and urban areas. As the predecessor of ENSANUT, the ENSA 2000 is the first cross-sectional survey in Mexico to include biological health metrics such as height and weight. In accordance with international guidelines, trained and standardized nurses measured these biomarkers on-site. Weight was measured in kilograms (kg) using a calibrated solar scale and height in centimeters (cm) using a flexometer. While the ENSANUT measures biomarkers for children over one year old, the ENSA 2000 only includes them for adults and children over ten years old (INSP, 2003).

In addition,[6] the ENSANUT includes data on the socioeconomic and demographic characteristics of household members, such as indigenous language, literacy, schooling, marital status, employment, self-reported income, and government subsidies. It also incorporates information on houses' economic characteristics, such as ownership, construction materials (e.g., floors, walls, roofs), number of rooms, household assets, sanitary conditions (e.g., drinking water, sewage), and access to and utilization of

[6]From now on, I will use ENSANUT to refer to both the ENSA (2000) and ENSANUT surveys.

healthcare services. Using these, I construct a household's socioeconomic status (SES) index — for each year —using principal component analysis (PCA). When comparing the index with Progresa's take-up rate in 2006, all beneficiary households lie in the first SES tercile. Thus, I define children from households in the lowest SES tercile in each wave who are more likely to be part of Progresa's eligible population. For a year-to-year comparison, I convert this index to percentiles.

By matching the two datasets, I construct a repeated cross-section database for individuals by birth cohort and place of residence. This includes cohorts born between 1983 and 1989 who would have been eligible to receive Progresa's education grant (ages 11 to 17) if their locality had been treated in 2000 (with available health data in the first wave). My sample includes 11,710 children residing in 427 urban localities. In the next section, I propose and test an identification strategy to estimate the intent-to-treat (ITT) effect of receiving Progresa during early adolescence.

4 Identification Strategy

My identification strategy exploits temporal variation in Progresa's roll-out at the local level and in age at first exposure to the program. For each locality ℓ, I observe the year they received the program for the first time. I focus the analysis on urban localities receiving Progresa between 2001 and 2005, which I define as the treatment adoption group G_ℓ. Note that this excludes 17 localities treated later, which are also very different from those treated during my period of interest (see Table 2).

However, not all children exposed to the intervention will be potentially affected. Progresa can only affect children's health biomarkers if it occurs during biological growth. For the same treatment adoption group G_ℓ, the individual's exposure to treatment will vary by birth cohort. For each cohort of birth j, I calculate the age at first exposure to Progresa based on their locality's treatment adoption group as: $\tilde{a}_0 = G_\ell - j$. Following Parker and Vogl (2023), I set fourteen as the maximum age of effective exposure to

Table 1: Mean Descriptive Statistics on Urban Households by Socioeconomic Status

	ENSANUT 2000			ENSANUT 2006		
	(1) All	(2) Low SES	(3) High SES	(4) All	(5) Low SES	(6) High SES
Take-up of Progresa (%)	0.0	0.0	0.0	9.8	26.5	0.0
Household size	3.98	4.76	3.67	4.28	4.33	4.28
Health Insurance (prop.)	0.58	0.43	0.65	0.61	0.56	0.64
With children (prop.)	0.68	0.90	0.60	0.69	0.70	0.69
Number of children	1.49	2.43	1.14	1.56	1.73	1.48
With adults over 70y (prop.)	0.13	0.03	0.15	0.15	0.20	0.13
Head of Household						
Age	45.7	36.4	48.0	48.3	49.5	47.7
Female (prop.)	0.04	0.04	0.04	0.22	0.29	0.19
Married (prop.)	0.76	0.88	0.72	0.76	0.69	0.79
Schooling (years)	7.76	6.68	8.21	7.30	5.28	8.58
House characteristics						
Rooms per person	0.61	0.36	0.71	0.58	0.51	0.62
Firm roof (prop.)	0.76	0.54	0.86	0.80	0.51	0.97
Firm floor (prop.)	0.96	0.88	0.99	0.96	0.89	1.00
Firm walls (prop.)	0.97	0.93	0.99	0.93	0.84	0.99
With electricity (prop.)	0.99	0.98	1.00	0.99	0.98	1.00
With sewage (prop.)	0.94	0.85	0.99	0.96	0.89	1.00
With water acces (prop.)	0.97	0.92	0.99	0.98	0.94	1.00
Households (N)	27,981	7,243	17,148	29,349	10,758	16,948

Notes: Sample weighted means. Low SES corresponds to the first index tercile; high SES includes the second and third index terciles. Sources: ENSANUT.

Progresa.[7] I define my treatment as a continuous variable that accounts for the total years of treatment received before (or equal to) 14 years old in 2006. Based on this definition, children must be younger than 14 years old in 2000. Furthermore, given that I observe only children over 10 years old at baseline, my analysis focuses on children born between 1987 and 1989, who were 11 to 13 years old in 2000 and 17 to 19 years old in 2006.

Similar to the analysis in Duflo (2001) in Indonesia, I employ a differences-in-differences (DiD) design with repeated cross-sectional data. In addition to the standard DiD estimation, I include a vector of birth cohorts by time-fixed effects that control for the stage in their growth trajectory. I estimate my model using the following equation:

[7]This threshold is also illustrated in Figure A.2, where growth stabilizes around fourteen years old for girls and around fifteen years for boys.

$$Y_{ij\ell st} \;=\; \beta\, Years\, Treated_{j\ell t} \,+\, \alpha_t \times \phi_j \,+\, \theta_{g(\ell)} + \mathbb{X}'_{i\ell}\Gamma \,+\, \eta_s \,+\, \varepsilon_{ij\ell st} \tag{1}$$

where, $Y_{ij\ell st}$ is the health outcome for individual i from cohort of birth j in locality ℓ in state s at year t; $Years\, Treated_{j\ell t}$ is the total years of treatment received before 14 years old in 2006; $\theta_{g(\ell)}$ corresponds to treatment adoption group fixed effects; α_t are time fixed effects; ϕ_j are cohort of birth fixed effects (1987-1989) $\mathbb{X}_{i\ell}$ is a vector of individual and locality covariates; η_s are state fixed effects; and $\varepsilon_{i\ell sct}$ is an error term. I cluster my standard errors by locality. Given that I cannot observe who was eligible to receive the program before it began, my analysis reflects an intent-to-treat (ITT) approach rather than a treatment effect on the treated.

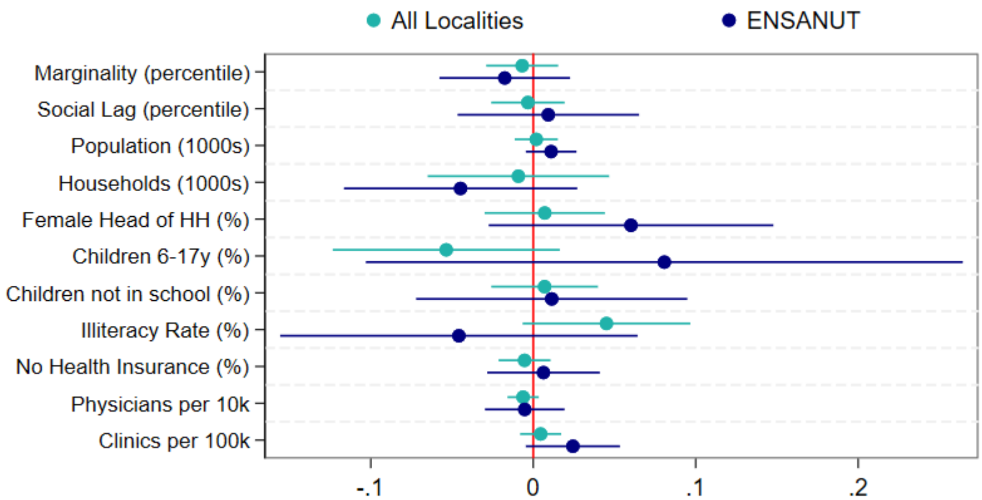
Table 2: Baseline Characteristics for Urban Localities by Treatment Adoption Group

	Means in 2000 by Year of First Treatment					
	(1) 2001	(2) 2002	(3) 2003	(4) 2004	(5) 2005	(6) After
Marginality (percentile)	59.3	42.5	32.7	37.1	37.4	18.3
Social Lag (percentile)	58.1	43.8	34.8	40.3	42.8	19.7
Population (1000s)	14.3	75.2	9.7	159.1	45.8	409.5
Households (1000s)	3.3	18.2	2.2	36.4	10.7	104.9
Female Head of HH (%)	18.5	18.1	15.9	16.1	14.9	20.8
Children 6-17y (%)	27.8	26.6	26.2	26.2	25.5	21.0
Children not in school (%)	17.3	14.9	14.9	15.4	13.1	15.7
Illiteracy Rate (%)	10.6	7.8	6.6	6.8	5.6	5.7
No Health Insurance (%)	65.4	55.6	52.1	57.0	61.8	54.0
Physicians per 10k	8.8	6.6	3.9	5.7	5.8	9.3
Clinics per 100k	12.2	8.0	12.2	8.2	8.4	2.4
Localities ($N = 1,166$)	615	349	13	130	47	17

Notes: Sample restricted to urban localities treated after 2000. Sources: CONAPO, INEGI, Progresa Administrative Records, Ministry of Health.

My identification strategy requires two main assumptions. First, it relies on the abrupt transition of urban localities into receiving Progresa. As mentioned before, given that the most impoverished municipalities received the intervention earlier, the second phase of Progresa's scale-up was less economically and geographically targeted. While the municipality's marginality index was the main criterion for incorporating new localities, there are no records of a clear threshold for determining their incorporation. By definition, urban localities have a lower marginality index than rural areas. Still, if initial poverty determines a locality's year of enrollment, my estimates will be biased. I test this by regressing a locality's transition year on a vector of pre-treatment demographic, socioeconomic, and geographic characteristics. Table 2 shows the descriptive statistics *pre*–intervention by treatment adoption group, along with the OLS coefficients. Despite differences in levels, I do not find evidence of a correlation between the timing of treatment and important economic determinants (columns 7 and 8). Another reason for this could be the closeness between urban centers, as proximity within treated localities was also an incorporation criterion to avoid migration between localities.

Figure 2: Predicting a Locality's Year of First Treatment (2001-2012)



Notes: $Year_{\ell m} = \alpha + \mathbb{X}'_{\ell}\beta + \eta_m + \varepsilon_{\ell m}$ for locality ℓ in municipality m. OLS coefficients with 95% confidence intervals (intercept omitted). Standard errors clustered by locality, with population weights. Sample restricted to urban localities treated after 2000. Sources: CONAPO, INEGI, Progresa Administrative Records, Ministry of Health, ENSANUT.

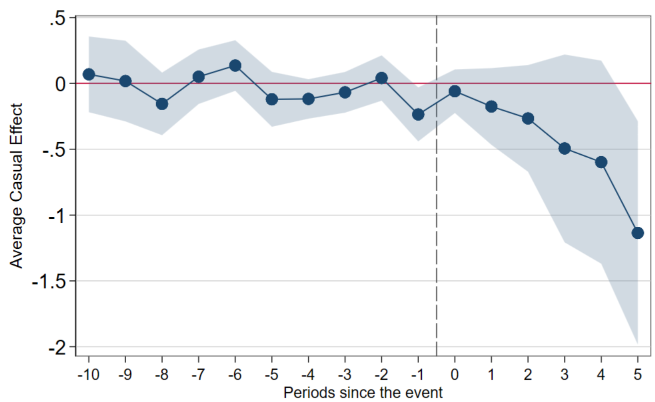
The second assumption refers to the parallel trends (PT) premise. My strategy assumes if treatment had not occurred, the average outcomes for all adoption groups g_ℓ would have evolved in parallel.[8] Ideally, I would test for pre-treatment differences between the treatment and control groups for my main outcomes. However, before 2000, Mexico lacked disaggregated data on children's health biomarkers. Instead, I use three outcomes as proxies for children's health, as the literature has shown they correlate with health biomarkers. Following Callaway and Sant'Anna (2021), I implement an event-study analysis by year when a locality first receives the program using annual fertility rates and

[8]For all $t \neq t'$ and $g \neq g'$: $\mathbb{E}\left[Y_{\ell,t}(0) - Y_{\ell,t'}(0) \mid G_\ell = g\right] = \mathbb{E}\left[Y_{\ell,t}(0) - Y_{\ell,t'}(0) \mid G_\ell = g'\right]$

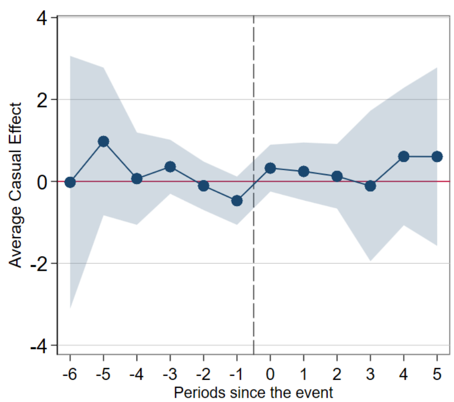
infant and neonatal mortality rates for urban localities (Figure 3). Consistent with previous work by Barham (2011), I find no significant effects of Progresa on infant mortality in urban areas, and this is also true for neonatal mortality rates. Regarding fertility rates, I observe a significant decrease after the intervention. However, given my sample was at least ten years old at the time of the intervention, this does not affect my analysis.

Figure 3: Event-Study Analysis for Pre-Trends in Health Outcomes

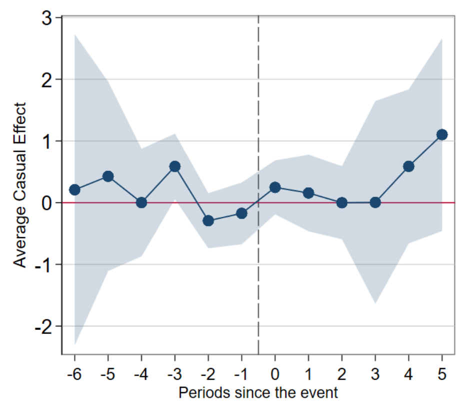
(a) Fertility Rate: 1990-2006



(b) Infant Mortality Rate: 1998-2006



(c) Neonatal Mortality Rate: 1998-2006



Notes: Callaway and Sant'Anna (2021) estimator with 95% confidence intervals (CI). Infant Mortality Rate equals the number of deaths in children under one year old per 1,000 births. The neonatal Mortality Rate equals the number of deaths in children under one month old per 1,000 births. Fertility Rate equals total births per 1,000 inhabitants. All rates are annual by urban locality of residence and weighted by population. H_0: $\boldsymbol{\beta}^{PRE} = 0$, $p-$value: (a) 0.500, (b) 0.883, (c) 0.837. Sources: Natality records, Mortality records, Population Censuses (1990, 1995, 2000, 2005, 2010).

5 Results

My results show the intent-to-treat effects of receiving one more year of Progresa before age fourteen. I find that Progresa significantly increases height and weight among children in urban areas. My estimates are robust to the inclusion of individual and locality controls (see Appendix B). Table 3 shows the estimates from my preferred specification, controlling for the socioeconomic index, the marginality index, the share of children aged 6-17, and the number of physicians per 1,000 population. Treated boys gain 0.42 centimeters (cm) in height per year of exposure to Progresa, corresponding to a 1.5% increase in height over an average period of 5 years. On the other hand, the ITT effect on girls' height is positive but not statistically significant (0.2 cm). This is consistent with their growth period ending earlier than that of boys, approximately 1 year after menarche (first menstruation; between 11 and 12 years old).

In addition, I find positive and significant effects on weight for both sexes. Urban children receiving Progresa before age fourteen gain between 0.63 and 0.79 kilograms (kg) more weight per year of treatment. This corresponds to an average 7.5% increase in weight over 5 years of exposure to the intervention among boys and a 9.2% increase among girls. Based on these, I only find significant ITT effects on girls' BMI. Girls gain 0.37 units of BMI ($^{kg}/_{m^2}$), which translates to a 1.8% increase per year of treatment. Though still positive for boys, the coefficient on BMI is less precise, as boys' weight gain is also accompanied by an increase in their height (a 0.6% increase per year of treatment). Nonetheless, these increments in weight and BMI are only beneficial for wasted and underweight children. Otherwise, more weight could be correlated with higher rates of overweight and obesity.

Following on these, table 4 shows the intent-to-treat effects on the probability of being underweight, overweight, and obese, each with respect to the normal weight BMI category. The first two columns show my estimates of underweight prevalence for boys and girls, respectively. I do not find any statistically significant effects on these, likely driven by a low prevalence of underweight in this population — on average, 4.4%. On the other side, I find positive and statistically significant effects on overweight and obesity, where around one-third of my sample is either overweight or obese. On average, receiving one more year of Progresa increases girls' overweight and obesity probability by 2.8 and 1.7 percentage points, respectively. In the case of boys, both estimates are more imprecise; a one percentage point increase in obesity prevalence is associated with one more year of receiving treatment.

Table 3: Intent-to-Treat Effects on Anthropometric Measures

	Height (cm)		Weight (kg)		BMI	
	(1) Boys	(2) Girls	(3) Boys	(4) Girls	(5) Boys	(6) Girls
Years Treated	0.424**	0.208	0.626**	0.789**	0.128	0.369***
	(0.179)	(0.164)	(0.272)	(0.317)	(0.093)	(0.120)
SES Index	0.042	0.030	0.083**	0.121***	0.026*	0.043***
	(0.027)	(0.022)	(0.039)	(0.036)	(0.015)	(0.014)
(SES Index) ^{2}	0.0001	-0.0001	-0.0005	-0.0011***	-0.0002	-0.0004***
	(0.0003)	(0.0002)	(0.0004)	(0.0004)	(0.0002)	(0.0002)
Locality Controls	yes	yes	yes	yes	yes	yes
State FE	yes	yes	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes
Mean DV	144.1	145.3	41.7	42.8	19.8	20.0
Observations	2,702	2,960	2,726	2,929	2,697	2,907
R^2	0.765	0.492	0.554	0.361	0.216	0.213

Notes: Sample restricted to birth cohorts from 1987 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights and standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

In addition, I repeat the analysis without any restrictions on the effective age at treatment (14 years old), including all birth cohorts eligible to receive Progresa for at least 1 year between 2000 and 2006 (i.e., children born between 1983 and 1989). Consistent with the fact that boys continue to grow until 18 years old, I find larger ITT effects of receiving one additional year of Progresa's intervention on both boys' height and weight (Table A.6). However, these increments are not accompanied by an increase in the prevalence of overweight and obesity, as before. On the other hand, though positive, I do not find any statistically significant effect on girls' anthropometric measures. Yet, their probability of being overweight increases by 3.7 percentage points, and their obesity prevalence by 2.2 percentage points (not statistically significant), with one more year of receiving Progresa (Table A.7).

Further, my estimates evidence a non-linear relationship between socioeconomic status and risk of overweight and obesity. While a higher SES index is associated with higher weight and BMI, the effects are smaller among individuals at the far end of the SES distribution. This is consistent with recent literature emphasizing how the urban poorest face a higher risk

Table 4: Intent-to-Treat Effects on BMI Categories

	Underweight		Overweight		Obesity	
	(1) Boys	(2) Girls	(3) Boys	(4) Girls	(5) Boys	(6) Girls
Years Treated	0.0089	-0.0020	0.0021	0.0280**	0.0104*	0.0167***
	(0.0086)	(0.0078)	(0.0124)	(0.0142)	(0.0063)	(0.0057)
SES Index	-0.0021	-0.0009	0.0029*	0.0031**	0.0016*	0.0020**
	(0.0016)	(0.0009)	(0.0015)	(0.0015)	(0.0009)	(0.0008)
(SES Index) ^{2}	0.00003	0.00001	-0.00003	-0.00003*	-0.00001	-0.00002*
	(0.00002)	(0.00001)	(0.00002)	(0.00002)	(0.00001)	(0.00001)
Locality Controls	yes	yes	yes	yes	yes	yes
State FE	yes	yes	yes	yes	yes	yes
Cohort × Time FE	yes	yes	yes	yes	yes	yes
Mean DV	0.042	0.046	0.278	0.257	0.078	0.052
Observations	1,792	2,022	2,339	2,647	1,872	2,026
R^2	0.053	0.030	0.034	0.040	0.038	0.048

Notes: Comparison group is normal weight. Sample restricted to birth cohorts from 1987 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights and standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p<0.01$, ** $p<0.05$, * $p<0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

of undernutrition and overweight, also known as the double burden of malnutrition (Popkin et al., 2020). To delve deeper into this, I perform a new analysis by socioeconomic status, interacting my treatment with a dummy variable for *Low SES* using the following equation:

$$\begin{aligned} Y_{ij\ell st} \; &= \beta_{_L} \, Years\,Treated_{j\ell t} \times Low_{it} \; + \; \beta_{_H} \, Years\,Treated_{j\ell t} \times High_{it} \\ &+ \alpha_t \times \phi_j \; + \theta_{_{g(\ell)}} + \mathbb{X}_{i\ell}' \Gamma \; + \; \eta_s \; + \varepsilon_{ij\ell st} \end{aligned} \tag{2}$$

where, $Low_{it} = 1$ if individual i at year t corresponds to the lowest tercile of the SES index distribution, and $High_{it} = 1$ if individual i at year t is above the first tercile of the SES index distribution. As before, my main estimates, $\hat{\beta}_{_L}$ and $\hat{\beta}_{_H}$, correspond to the intent-to-treat effect of receiving one more year of treatment –for each income group, respectively.

Figure 4 shows these estimates by sex, comparing them with my results from model 1 (dotted line). First, all estimates for low-SES children are statistically different from zero (except for underweight prevalence), whereas only the coefficient on BMI for high-SES girls remains statistically significant. This suggests that most of the previously observed effect is driven by children in the lowest socioeconomic tercile, as only low-income households were eligible for Progresa.

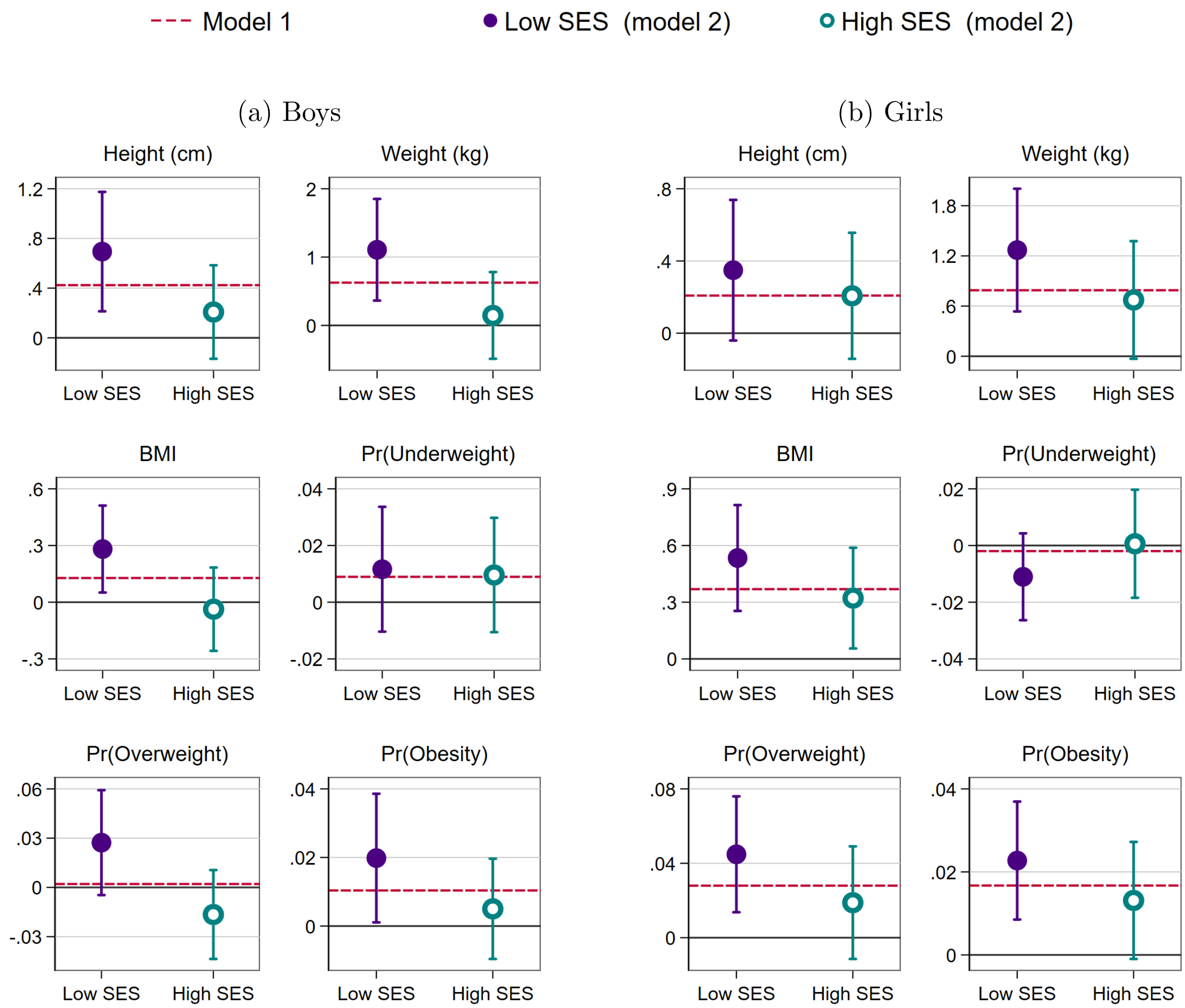
On average, for each additional year of exposure to Progresa, poor urban boys gain 0.69 cm in height, and poor urban girls gain 0.35 cm. Similarly, poor urban children receiving Progresa gain around 1.2 kilograms (kg) more weight per year of treatment, which also translates to a significant increase in BMI (0.3 and 0.5 units for boys and girls, respectively). However, these BMI increments have different interpretations between boys and girls. On one side, some of the increase in BMI is helping to reduce underweight prevalence among low SES girls (though not statistically significant). On the other side, poor treated girls increase their overweight prevalence by 4.5 percentage points for each year of receiving Progresa treatment and have 2.3 percentage points more probability of being obese. In the case of poor-treated boys, only the prevalence of overweight and obesity increases by 2.7 and 2.0 percentage points, respectively.

Note that all these intent-to-treat effects are significantly larger when we calculate the local average treatment effect (LATE) using the program's observed 2006 take-up rate among low-SES children (26.5%). For example, the LATE for poor urban boys corresponds to a 2.6 cm increase in height for each year of treatment received before age 14, and 1.3 cm among poor urban girls. Alternatively, I can estimate the LATE using a Two-Step Least Squares (2SLS) approach, using Progresa's 2006 take-up rate as an instrument (Imbens and Angrist, 1994). Table 5 summarizes these local average treatment effects on each outcome by sex, using both methodologies. The LATE estimates are qualitatively similar, though the 2SLS are more conservative.

6 Discussion and Policy Implications

Compared to the RCT effects on children's health, this study finds substantially larger effects of Progresa on boys' height—approximately double those observed in the original rural trial—and similar effects for girls (Gertler, 2004; Fernald et al., 2009, 2008b). The

Figure 4: Intent-to-Treat Effects of Progresa by Sex and Socioeconomic Status



Notes: $\widehat{\beta}_s$ coefficient by SES group with 95% confidence intervals (equation 2). Dotted line shows the coefficient from equation 1. Sample restricted to children born between 1987-1989 from urban localities treated between 2001-2005. All regressions include sample weights and standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

timing of intervention and biological differences by sex may partially explain these variations, but they do not account for the increased risk of overweight and obesity among urban poor children. These apparently divergent effects highlight the importance of considering the unique characteristics of urban beneficiaries when assessing the overall impact of Progresa on children's health.

Progresa was originally designed for rural communities, where the cash transfer was meant to offset the opportunity cost of child labor. In contrast, even among low-income households, urban areas tend to have higher educational attainment, potentially reducing the trade-off

Table 5: Local Average Treatment Effects by Additional Year of Treatment

	OLS		2SLS	
	(1)	(2)	(3)	(4)
Outcome	Boys	Girls	Boys	Girls
Height (cm)	2.013***	1.012*	1.939**	0.810
	(0.709)	(0.574)	(0.855)	(0.755)
Weight (kg)	3.212***	3.683***	2.853**	3.162**
	(1.097)	(1.083)	(1.316)	(1.485)
BMI ($^{kg}/_{m^2}$)	0.816**	1.549***	0.560	1.557***
	(0.339)	(0.413)	(0.446)	(0.564)
Pr(Underweight)	0.0337	-0.0320	0.0377	-0.0075
	(0.0325)	(0.0226)	(0.0415)	(0.0369)
Pr(Overweight)	0.0791*	0.1300***	0.0001	0.1168*
	(0.0471)	(0.0460)	(0.0588)	(0.0661)
Pr(Obesity)	0.0575**	0.0659***	0.0452	0.0718***
	(0.0277)	(0.0210)	(0.0303)	(0.0274)

Notes: Sample restricted to birth cohorts from 1987 to 1989 from urban localities treated between 2001-2005. *** $p<0.01$, ** $p<0.05$, * $p<0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

between receiving Progresa's CCT and school participation. Prior research on Progresa's impact in urban settings confirms this. Behrman et al. (2012) report that an average of 18 months of treatment increases secondary school enrollment by 2.7–3.0 percentage points for girls and 1.0–1.3 percentage points for boys aged 12 to 14. By comparison, the rural RCT showed much larger effects for the same age group: 9 and 6 percentage points for girls and boys, respectively (Schultz, 2004).

These patterns suggest a higher marginal benefit per dollar transferred among urban households, even though food access constraints are less severe. Urban beneficiaries spend a substantial share (around 80%) of transfers on food (Angelucci and Attanasio, 2009), resulting in a 12.0–17.5% increase in total calorie consumption after 18 months, compared to a 7% increase in the rural RCT (Gertler et al., 2012; Hoddinott and Skoufias, 2004). While this increased consumption may contribute to greater height gains in urban children, it may also lead to unintended consequences, since there are no restrictions on how transfers are spent.

The risk of increased BMI and obesity from cash transfers has been documented among adults in the RCT (Fernald et al., 2008a) and in other programs (Levasseur, 2019; Forde et al., 2012), but my findings indicate that Mexican urban children experience these risks at earlier ages. Despite the perceived urban advantage, the poorest in urban areas face significant barriers to healthy diets, including higher transportation costs, limited access to fresh foods, and a lack of self-production (Vilar-Compte et al., 2021; Dutra et al., 2018). Combined with the proliferation of ultra-processed foods, these factors heighten the risk of overweight and obesity among urban adolescents. These findings call for policy adaptations to CCT program design in urban contexts and further research to better understand how households allocate transfers and the constraints they face in accessing nutritious food.

7 Conclusion

This study provides new causal evidence on the effects of Mexico's Progresa conditional cash transfer program on children's health in urban settings. Unlike the original rural experiment, Progresa's expansion to urban areas produced heterogeneous impacts: while income transfers led to substantial gains in height and weight, especially among boys and low-SES children—they also increased the risk of overweight and obesity, notably among girls and the poorest households. These results underscore the complexity of addressing malnutrition in rapidly urbanizing environments, where poverty and exposure to unhealthy food environments coexist.

This study is not without limitations. First, it uses repeated cross-sectional rather than longitudinal data, making it impossible to follow individual children over time or directly observe growth trajectories. This could lead to compositional changes across survey waves. Second, the health surveys include only a subset of urban localities, so the results may not generalize to all areas that received Progresa. Third, outcomes are only observed through 2006, limiting the analysis to medium-term effects during adolescence and leaving the long-term impacts unknown. Fourth, the estimates reflect Progresa's urban expansion in the early 2000s. Later versions of the program—Oportunidades and Prospera—differed in benefit levels, operations, and targeting, so these findings may not apply to those iterations. Additionally, pre-program eligibility cannot be directly observed, so effects are estimated at the locality level rather than for individual beneficiaries. Finally, because the study cannot separate the impact of different program components, the estimated effects represent the combined influence of cash transfers, nutritional supplements, health-care requirements, and workshops.

Despite these limitations, this paper offers new evidence on Progresa's urban expansion and the trade-offs of cash transfer programs in cities. The results show that while CCTs can promote growth and human capital during adolescence, they may also worsen the double burden of malnutrition. Effective policy should tailor CCTs to urban contexts and consider complementary strategies—like nutrition education, healthy food incentives, and better access to fresh foods—to maximize benefits and minimize health risks. Future research should examine the mechanisms behind these patterns and evaluate program adaptations that address the unique nutritional needs of urban youth.

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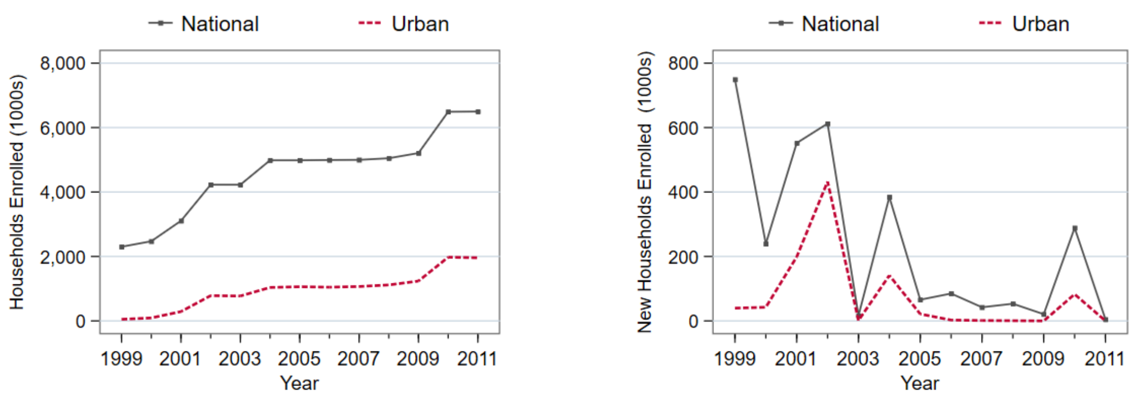
Appendix

A Figures

Figure A.1: Progresa New Household Enrollment, 1999-2011

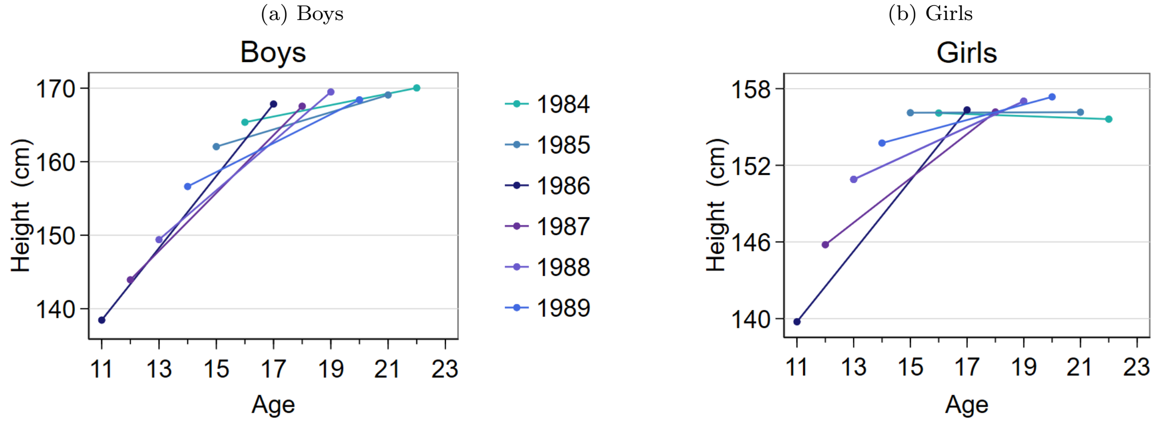
(a) Total Households (b) New Households



Source: Progresa Administrative Records.

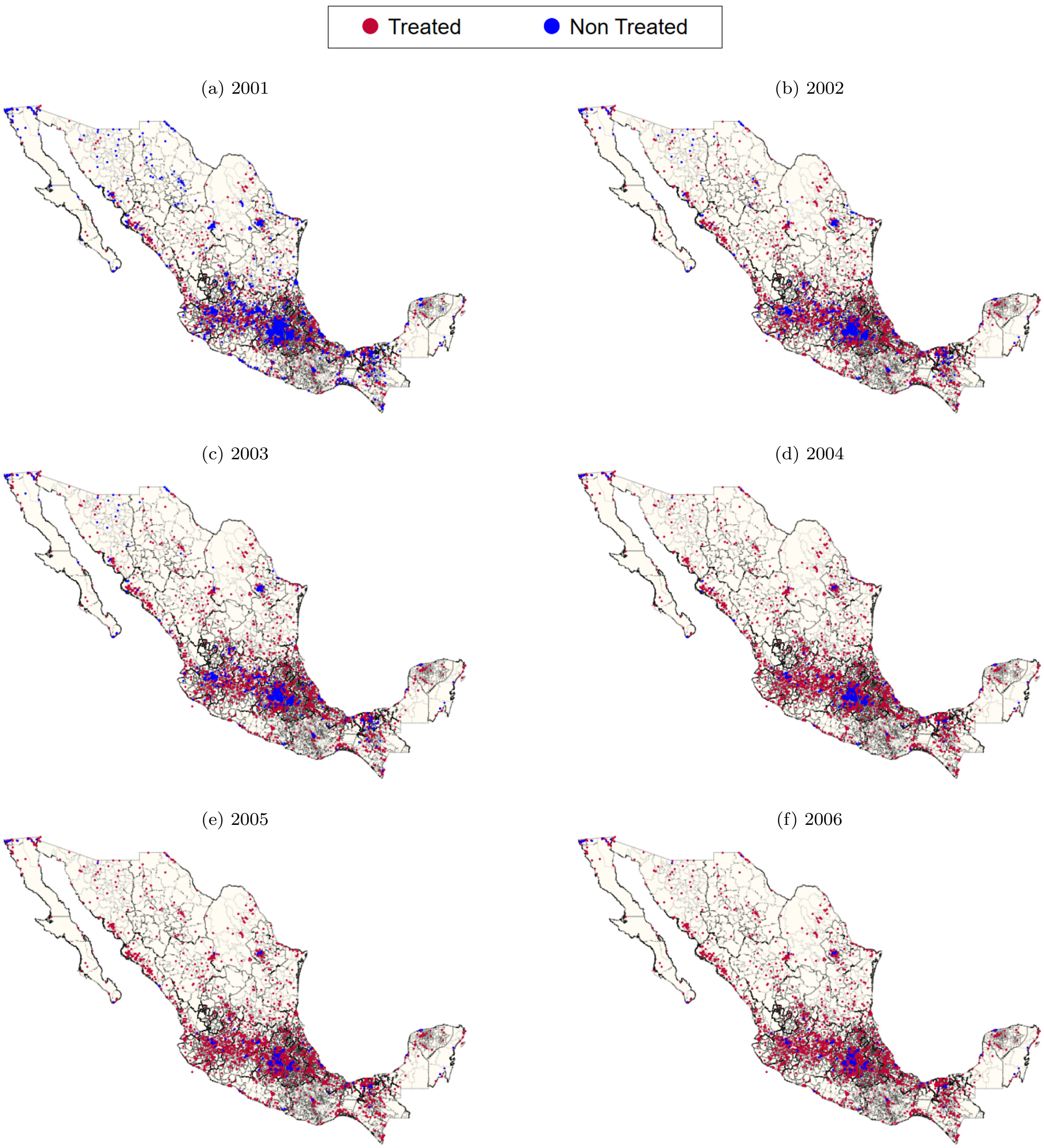
Figure A.2: Children's Height Trajectory by Sex and Cohort of Birth

(a) Boys (b) Girls



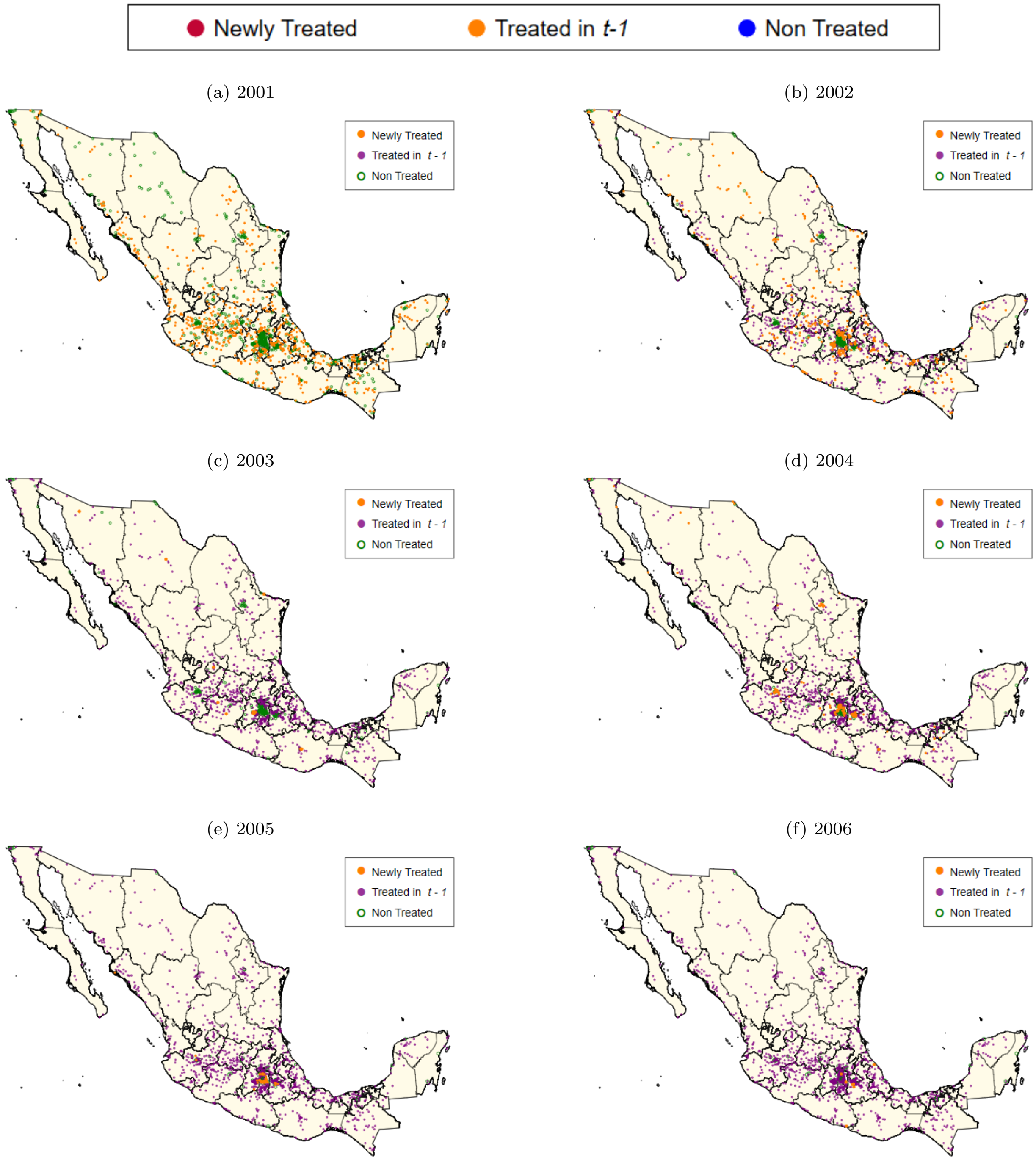
Notes: Mean height in each wave (2000 and 2006) by cohort of birth with their age (horizontal axis) . Sample restricted to children from urban localities treated between 2001-2006. Sources: ENSANUT, Progresa Administrative Records.

Figure A.3: Progresa's Scaling-Up to Urban Localities by Year, 2001-2006



Source: Progresa Administrative Records.

Figure A.4: Progresa's Scaling-Up to New Urban Localities by Year of Entrance, 2001-2006



Source: Progresa Administrative Records.

B Tables

Table A.1: Descriptive Statistics on Urban Households by Socioeconomic Status

	ENSANUT 2000			ENSANUT 2006		
	(1) All	(2) Low SES	(3) High SES	(4) All	(5) Low SES	(6) High SES
Take-up of Progresa (%)	0.0	0.0	0.0	9.8	26.5	0.0
	(0.0)	(0.0)	(0.0)	(29.7)	(44.2)	(0.0)
Household size	3.98	4.76	3.67	4.28	4.33	4.28
	(1.87)	(1.98)	(1.68)	(1.94)	(2.12)	(1.84)
Health Insurance (prop.)	0.58	0.43	0.65	0.61	0.56	0.64
	(0.49)	(0.50)	(0.48)	(0.49)	(0.50)	(0.48)
With children (prop.)	0.68	0.90	0.60	0.69	0.70	0.69
	(0.47)	(0.30)	(0.49)	(0.46)	(0.46)	(0.46)
Number of children	1.49	2.43	1.14	1.56	1.73	1.48
	(1.43)	(1.56)	(1.17)	(1.45)	(1.60)	(1.35)
With adults over 70y (prop.)	0.13	0.03	0.15	0.15	0.20	0.13
	(0.33)	(0.16)	(0.35)	(0.36)	(0.40)	(0.33)
Head of Household						
Age	45.7	36.4	48.0	48.3	49.5	47.7
	(16.1)	(11.8)	(15.9)	(15.3)	(16.8)	(14.5)
Female (prop.)	0.04	0.04	0.04	0.22	0.29	0.19
	(0.19)	(0.21)	(0.19)	(0.42)	(0.45)	(0.39)
Married (prop.)	0.76	0.88	0.72	0.76	0.69	0.79
	(0.43)	(0.33)	(0.45)	(0.43)	(0.46)	(0.40)
Schooling (years)	7.76	6.68	8.21	7.30	5.28	8.58
	(4.42)	(3.60)	(4.64)	(4.46)	(3.86)	(4.34)
House characteristics						
Rooms per person	0.61	0.36	0.71	0.58	0.51	0.62
	(0.42)	(0.18)	(0.45)	(0.37)	(0.35)	(0.37)
Firm roof (prop.)	0.76	0.54	0.86	0.80	0.51	0.97
	(0.42)	(0.50)	(0.34)	(0.40)	(0.50)	(0.17)
Firm floor (prop.)	0.96	0.88	0.99	0.96	0.89	1.00
	(0.20)	(0.33)	(0.08)	(0.20)	(0.32)	(0.00)
Firm walls (prop.)	0.97	0.93	0.99	0.93	0.84	0.99
	(0.16)	(0.25)	(0.07)	(0.26)	(0.37)	(0.11)
With electricity (prop.)	0.99	0.98	1.00	0.99	0.98	1.00
	(0.09)	(0.14)	(0.04)	(0.09)	(0.15)	(0.00)
With sewage (prop.)	0.94	0.85	0.99	0.96	0.89	1.00
	(0.23)	(0.36)	(0.11)	(0.20)	(0.32)	(0.00)
With water acces (prop.)	0.97	0.92	0.99	0.98	0.94	1.00
	(0.17)	(0.28)	(0.08)	(0.15)	(0.23)	(0.02)
Households (N)	27,981	7,243	17,148	29,349	10,758	16,948

Notes: Sample weighted means with standard deviation in parenthesis below. Low SES corresponds to first index tercile; high SES includes second and third index terciles. Sources: ENSANUT.

Table A.2: Descriptive Statistics on Anthropometric Measures

	Male			Female		
	ENSANUT 2000	ENSANUT 2006	p-value	ENSANUT 2000	ENSANUT 2006	p-value
Adults						
Height (cm)	166.9	166.9	0.929	154.3	154.2	0.287
Weight (kg)	77.2	78.2	0.002	67.8	69.2	0.000
BMI	27.5	27.9	0.000	28.3	29.0	0.000
Underweight (%)	0.8	0.8	0.963	0.9	0.7	0.098
Overweight (%)	46.1	46.2	0.875	39.0	38.1	0.250
Obesity (%)	25.7	28.2	0.012	33.5	38.5	0.000
Observations	3,684	4,527	8,211	7,858	7,193	15,051
Cohorts: 1983-1989						
Height (cm)	153.9	168.8	0.000	150.9	156.6	0.000
Weight (kg)	50.2	69.3	0.000	49.6	59.4	0.000
BMI	20.8	24.3	0.000	21.5	24.1	0.000
Underweight (%)	5.7	6.2	0.479	5.8	5.4	0.544
Overweight (%)	20.5	24.8	0.000	23.8	24.4	0.633
Obesity (%)	9.2	11.1	0.021	8.7	9.0	0.647
Observations	3,108	2,414	5,522	3,335	2,853	6,188

Notes: Sample weighted means. Adults includes individuals between 25 to 49 years old. Sources: ENSANUT.

Table A.3: Descriptive Statistics on Progresa's Beneficiary Households

	ENSANUT 2000		ENSANUT 2006				
	Rural		Rural		Urban		Mean
	Mean	SD	Mean	SD	Mean	SD	Differences
Household size	5.06	(2.23)	4.83	(2.14)	5.28	(2.21)	0.458***
With children (prop.)	0.84	(0.37)	0.81	(0.40)	0.88	(0.33)	0.070***
Number of children	2.68	(1.95)	2.33	(1.78)	2.69	(1.73)	0.356***
Head of Household							
Age	45.5	(15.3)	47.9	(15.5)	45.4	(14.4)	-2.456***
Female (prop.)	0.03	(0.18)	0.20	(0.40)	0.24	(0.43)	0.046***
Married (prop.)	0.86	(0.35)	0.83	(0.37)	0.79	(0.41)	-0.047***
Schooling (years)	4.23	(2.78)	4.03	(3.23)	4.63	(3.47)	0.605***
House characteristics							
Rooms per person	0.39	(0.27)	0.43	(0.26)	0.37	(0.21)	-0.059***
Firm roof (prop.)	0.38	(0.49)	0.44	(0.50)	0.53	(0.50)	0.092***
Firm floor (prop.)	0.61	(0.49)	0.77	(0.42)	0.83	(0.37)	0.058***
Firm walls (prop.)	0.91	(0.28)	0.79	(0.41)	0.87	(0.34)	0.073***
With electricity (prop.)	0.90	(0.30)	0.95	(0.22)	0.98	(0.14)	0.029***
With sewage (prop.)	0.40	(0.49)	0.69	(0.46)	0.88	(0.33)	0.185***
With water acces (prop.)	0.62	(0.49)	0.81	(0.40)	0.95	(0.23)	0.139***
Households (N)	4,195		10,257		3,151		13,408

Notes: Sample restricted to urban localities treated after 2000 in ENSANUT. No treated localities on 2003 appear in data. Sources: CONAPO, INEGI, Progresa Administrative Records, Ministry of Health, ENSANUT.

Table A.4: Baseline Characteristics for Urban Localities (ENSANUT Sample)

	Means in 2000 by Year of First Treatment				
	(1)	(2)	(3)	(4)	(5)
	2001	2002	2004	2005	After
Marginality (percentile)	54.9	34.3	25.6	24.9	4.9
Social Lag (percentile)	53.5	34.0	29.3	29.7	4.9
Population (1000s)	17.2	129.2	341.5	174.1	688.1
Households (1000s)	4.0	31.5	78.7	41.8	176.7
Female Head of HH (%)	17.8	18.6	16.4	16.6	24.3
Children 6-17y (%)	27.5	26.2	25.2	24.3	19.7
Children not in school (%)	16.5	14.6	13.4	11.3	8.8
Illiteracy Rate (%)	9.4	6.8	5.0	4.5	2.4
No Health Insurance (%)	62.3	50.8	50.5	57.6	45.9
Physicians per 10k	9.4	7.4	6.4	10.5	15.0
Clinics per 100k	11.8	6.7	7.6	5.8	2.3
Localities ($N = 427$)	181	175	54	9	10

Notes: Sample restricted to urban localities treated after 2000 in ENSANUT. No treated localities on 2003 appear in data. Sources: CONAPO, INEGI, Progresa Administrative Records, Ministry of Health, ENSANUT.

Table A.5: Descriptive Statistics in 2000 by Progresa's Scale-up Phase and Type of Locality

	Urban						Rural					
	(1) 1999-2000		(2) 2001-2005		(3) After 2006		(4) 1999-2000		(5) 2001-2005		(6) After 2006	
	Mean	SD	Mean	SD	Mean	SD	Mean	SD	Mean	SD	Mean	SD
Mun. Marginality	56.2	21.0	32.5	21.1	5.9	6.9	57.5	24.4	51.7	28.6	55.1	29.4
Population (1000s)	9.7	13.8	50.3	142.0	387.0	482.4	0.40	0.63	0.36	0.84	0.09	0.26
Households (100s)	21.1	34.7	118.2	334.4	991.4	1178.2	0.84	1.36	0.81	1.90	0.19	0.57
Members per house	4.7	0.5	4.3	0.4	4.1	0.7	4.7	0.9	4.7	1.2	4.6	1.3
Pop. Density	3.1	4.2	2.8	7.8	0.1	0.1	5.5	11.1	7.0	16.1	7.6	19.4
Female (%)	51.2	1.3	51.5	1.2	49.5	11.5	50.2	5.3	49.6	7.1	48.7	8.5
Children 6-17y (%)	30.0	2.8	27.2	2.5	21.1	6.8	30.5	7.6	28.3	10.1	27.4	11.7
Illiteracy Rate (%)	19.3	10.5	9.1	5.1	5.6	9.4	22.6	15.0	21.5	18.8	25.3	22.2
Schooling (years)	5.5	1.3	7.2	1.2	8.8	2.2	4.2	1.3	4.4	1.9	4.1	2.0
No Healthcare (%)	80.1	16.3	61.2	17.3	52.6	20.3	86.4	20.4	81.7	24.0	82.6	26.7
Physicians per 100k	43.1	47.6	60.0	71.9	66.1	101.7	21.4	152.5	25.0	533.1	6.9	160.5
Clinics per 100k	10.9	9.2	10.3	10.5	2.3	3.1	18.9	135.6	17.2	220.0	6.5	157.4
Hospitals per million	4.8	21.7	8.6	25.2	2.9	4.2	0.3	16.6	1.9	151.0	0.4	41.1
Geographic Region												
North (%)	17.3	38.0	18.7	39.0	5.6	23.6	18.7	39.0	24.1	42.8	28.4	45.1
Center (%)	39.8	49.1	53.9	49.9	94.4	23.6	41.2	49.2	43.6	49.6	26.7	44.2
South (%)	42.9	49.7	27.5	44.6	0.0	0.0	40.1	49.0	32.3	46.8	45.0	49.7
Localities (N)	134		1,154		18		18,351		26,945		17,699	

Notes: Means and standard deviations by locality. Urban localities have 5,000 inhabitants or more. Municipality marginality index expressed in percentiles, population density refers to mean by municipality. Sources: CONAPO, INEGI, Progresa Administrative Records, Ministry of Health.

Table A.6: Intent-to-Treat Effects on Anthropometric Measures (Cohorts: 1983-1989)

	Height (cm)		Weight (kg)		BMI	
	(1) Boys	(2) Girls	(3) Boys	(4) Girls	(5) Boys	(6) Girls
Years Treated	0.875***	0.216	1.246***	0.279	0.222	0.108
	(0.254)	(0.242)	(0.447)	(0.477)	(0.175)	(0.180)
SES Index	0.052***	0.023	0.057*	0.083***	0.015	0.027**
	(0.020)	(0.016)	(0.033)	(0.028)	(0.011)	(0.010)
(SES Index) ^{2}	-0.0001	0.0000	-0.0002	-0.0007**	-0.0001	-0.0003**
	(0.0002)	(0.0002)	(0.0004)	(0.0003)	(0.0001)	(0.0001)
Locality Controls						
Marginality	0.001	-0.026**	0.021	-0.006	0.010	0.004
(percentile)	(0.014)	(0.012)	(0.019)	(0.015)	(0.007)	(0.007)
Children 6-17y (%)	-0.266**	-0.151	-0.388*	-0.263*	-0.105	-0.056
	(0.118)	(0.109)	(0.205)	(0.157)	(0.084)	(0.067)
Physicians per 1000s	0.063	-0.237	0.067	-0.170	0.068	0.024
	(0.195)	(0.221)	(0.308)	(0.232)	(0.105)	(0.123)
State FE	yes	yes	yes	yes	yes	yes
Cohort × Time FE	yes	yes	yes	yes	yes	yes
Mean DV	154.0	150.9	50.3	49.5	20.8	21.5
Observations	5,254	5,913	5,298	5,823	5,242	5,760
R^2	0.678	0.388	0.462	0.268	0.210	0.160

Notes: Sample restricted to birth cohorts from 1983 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.7: Intent-to-Treat Effects on BMI Categories (Cohorts: 1983-1989)

	Underweight		Overweight		Obesity	
	(1) Boys	(2) Girls	(3) Boys	(4) Girls	(5) Boys	(6) Girls
Years Treated	-0.0006	0.0105	0.0240	0.0367*	0.0157	0.0219
	(0.0127)	(0.0118)	(0.0184)	(0.0190)	(0.0107)	(0.0144)
SES Index	-0.0009	-0.0006	0.0006	0.0026**	-0.0000	0.0012*
	(0.0010)	(0.0007)	(0.0013)	(0.0011)	(0.0008)	(0.0007)
(SES Index) ^{2}	0.00001	0.00001	-0.00000	-0.00003**	0.00000	-0.00001
	(0.00001)	(0.00001)	(0.00001)	(0.00001)	(0.00001)	(0.00001)
Locality Controls						
Marginality	-0.0001	0.0009	0.0007	0.0007	0.0004	0.0010**
(percentile)	(0.0005)	(0.0007)	(0.0008)	(0.0007)	(0.0005)	(0.0005)
Children 6-17y (%)	-0.0087**	-0.0018	-0.0133	0.0011	-0.0092	-0.0070
	(0.0038)	(0.0049)	(0.0098)	(0.0063)	(0.0058)	(0.0056)
Physicians per 1000s	-0.0002	0.0053	-0.0023	-0.0070	0.0094	-0.0140
	(0.0094)	(0.0065)	(0.0120)	(0.0113)	(0.0090)	(0.0088)
State FE	yes	yes	yes	yes	yes	yes
Cohort × Time FE	yes	yes	yes	yes	yes	yes
Mean DV	0.064	0.055	0.330	0.341	0.095	0.070
Observations	3,517	3,886	4,977	5,520	3,679	3,964
R^2	0.032	0.039	0.047	0.041	0.058	0.092

Notes: Comparison group is normal weight. Sample restricted to birth cohorts from 1983 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. $***p<0.01$, $**p<0.05$, $*p<0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.8: Intent-to-Treat Effects on Boys' Height

	Boys' Height (cm)							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.380*	0.360*	0.365*	0.368*	0.423**	0.424**	0.405**	0.405**
	(0.211)	(0.211)	(0.201)	(0.195)	(0.179)	(0.179)	(0.177)	(0.178)
Age		1.061**						
		(0.513)						
SES Index			0.053***	0.049***	0.049***	0.042	0.043***	0.046*
(percentile)			(0.008)	(0.008)	(0.008)	(0.027)	(0.008)	(0.026)
$(\text{SES Index})^2$						0.0001		-0.0000
						(0.0003)		(0.0003)
Mother's Educ $\geq$ 6y							1.398***	1.406***
							(0.417)	(0.413)
Locality Controls								
Marginality				-0.019	0.036**	0.036**	0.039**	0.039**
(percentile)				(0.013)	(0.017)	(0.017)	(0.017)	(0.017)
Children 6-17y (%)				-0.059	-0.439***	-0.439***	-0.434***	-0.434***
				(0.139)	(0.155)	(0.155)	(0.154)	(0.154)
Physicians per 1000s				0.512	0.453	0.456	0.445	0.444
				(0.420)	(0.376)	(0.375)	(0.379)	(0.378)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	144.1	144.1	144.1	144.1	144.1	144.1	144.1	144.1
Observations	2,702	2,702	2,702	2,702	2,702	2,702	2,702	2,702
R^2	0.744	0.744	0.753	0.755	0.765	0.765	0.767	0.767

Notes: Sample restricted to birth cohorts from 1987 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.9: Intent-to-Treat Effects on Boys' Standardized Height for Age

	Boys' Height-for-Age (z-score)							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.045	0.048	0.043	0.043	0.050**	0.050**	0.048*	0.048*
	(0.030)	(0.030)	(0.028)	(0.027)	(0.025)	(0.025)	(0.025)	(0.025)
Age		-0.145**						
		(0.065)						
SES Index			0.007***	0.007***	0.007***	0.006*	0.006***	0.007**
(percentile)			(0.001)	(0.001)	(0.001)	(0.004)	(0.001)	(0.004)
(SES Index) ^{2}						0.0000		-0.0000
						(0.0000)		(0.0000)
Mother's Educ $\geq$ 6y							0.198***	0.202***
							(0.054)	(0.054)
Locality Controls								
Marginality				-0.003	0.005**	0.005**	0.005**	0.005**
(percentile)				(0.002)	(0.002)	(0.002)	(0.002)	(0.002)
Children 6-17y (%)				-0.017	-0.067***	-0.067***	-0.066***	-0.066***
				(0.020)	(0.022)	(0.022)	(0.021)	(0.021)
Physicians per 1000s				0.068	0.058	0.058	0.057	0.056
				(0.046)	(0.041)	(0.041)	(0.042)	(0.042)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	-0.34	-0.34	-0.34	-0.34	-0.34	-0.34	-0.34	-0.34
Observations	2,702	2,702	2,702	2,702	2,702	2,702	2,702	2,702
R^2	0.085	0.088	0.120	0.129	0.167	0.167	0.174	0.174

Notes: Sample restricted to birth cohorts from 1987 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.10: Intent-to-Treat Effects on Girls' Height

	Girls' Height (cm)							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.192	0.178	0.189	0.208	0.212	0.208	0.204	0.192
	(0.176)	(0.178)	(0.178)	(0.170)	(0.163)	(0.164)	(0.165)	(0.165)
Age		1.640***						
		(0.443)						
SES Index			0.037***	0.023***	0.023***	0.030	0.017***	0.039*
(percentile)			(0.007)	(0.006)	(0.006)	(0.022)	(0.006)	(0.021)
(SES Index) ^{2}						-0.0001		-0.0002
						(0.0002)		(0.0002)
Mother's Educ $\geq$ 6y							1.604***	1.665***
							(0.413)	(0.419)
Locality Controls								
Marginality				-0.069***	-0.039***	-0.038***	-0.035**	-0.033**
(percentile)				(0.012)	(0.015)	(0.015)	(0.014)	(0.014)
Children 6-17y (%)				0.124	-0.113	-0.111	-0.113	-0.107
				(0.107)	(0.145)	(0.146)	(0.140)	(0.141)
Physicians per 1000s				-0.151	-0.148	-0.144	-0.160	-0.148
				(0.265)	(0.288)	(0.287)	(0.273)	(0.273)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort × Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	145.3	145.3	145.3	145.3	145.3	145.3	145.3	145.3
Observations	2,960	2,960	2,960	2,960	2,960	2,960	2,960	2,960
R^2	0.443	0.447	0.455	0.472	0.492	0.492	0.498	0.499

Notes: Sample restricted to birth cohorts from 1987 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.11: Intent-to-Treat Effects on Girls' Standardized Height for Age

	Girls' Height-for-Age (z-score)							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.017	0.017	0.017	0.020	0.018	0.017	0.016	0.015
	(0.027)	(0.027)	(0.027)	(0.026)	(0.025)	(0.025)	(0.025)	(0.025)
Age		-0.044						
		(0.064)						
SES Index			0.005***	0.003***	0.003***	0.004	0.002***	0.005
(percentile)			(0.001)	(0.001)	(0.001)	(0.003)	(0.001)	(0.003)
(SES Index) ^{2}						-0.0000		-0.0000
						(0.0000)		(0.0000)
Mother's Educ $\geq$ 6y							0.233***	0.240***
							(0.062)	(0.063)
Locality Controls								
Marginality				-0.010***	-0.006***	-0.006***	-0.005**	-0.005**
(percentile)				(0.002)	(0.002)	(0.002)	(0.002)	(0.002)
Children 6-17y (%)				0.012	-0.020	-0.020	-0.020	-0.020
				(0.016)	(0.022)	(0.022)	(0.021)	(0.021)
Physicians per 1000s				-0.032	-0.028	-0.028	-0.030	-0.028
				(0.039)	(0.043)	(0.043)	(0.040)	(0.040)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	-0.31	-0.31	-0.31	-0.31	-0.31	-0.31	-0.31	-0.31
Observations	2,960	2,960	2,960	2,960	2,960	2,960	2,960	2,960
R^2	0.111	0.111	0.130	0.160	0.190	0.191	0.200	0.201

Notes: Sample restricted to birth cohorts from 1987 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.12: Intent-to-Treat Effects on Boys' Weight

	Boys' Weight (kg)							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.545**	0.524*	0.532*	0.534**	0.633**	0.626**	0.618**	0.608**
	(0.273)	(0.271)	(0.272)	(0.268)	(0.275)	(0.272)	(0.276)	(0.273)
Age		1.133						
		(0.817)						
SES Index			0.042***	0.039***	0.042***	0.083**	0.036***	0.088**
(percentile)			(0.011)	(0.011)	(0.011)	(0.039)	(0.011)	(0.039)
(SES Index) ^{2}						-0.0005		-0.0006
						(0.0004)		(0.0004)
Mother's Educ $\geq$ 6y							1.201**	1.332**
							(0.541)	(0.543)
Locality Controls								
Marginality				-0.011	0.034	0.034	0.037	0.037
(percentile)				(0.019)	(0.024)	(0.024)	(0.025)	(0.025)
Children 6-17y (%)				-0.150	-0.262	-0.264	-0.256	-0.259
				(0.162)	(0.208)	(0.209)	(0.207)	(0.207)
Physicians per 1000s				0.241	0.291	0.273	0.283	0.261
				(0.353)	(0.324)	(0.325)	(0.322)	(0.323)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	41.7	41.7	41.7	41.7	41.7	41.7	41.7	41.7
Observations	2,726	2,726	2,726	2,726	2,726	2,726	2,726	2,726
R^2	0.538	0.538	0.542	0.543	0.554	0.554	0.555	0.555

Notes: Sample restricted to birth cohorts from 1987 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.13: Intent-to-Treat Effects on Boys' Standardized Weight for Age

	Boys' Weight-for-Age (z-score)							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.038	0.040	0.036	0.036	0.048*	0.047*	0.046*	0.045*
	(0.030)	(0.030)	(0.029)	(0.028)	(0.028)	(0.027)	(0.028)	(0.027)
Age		-0.138*						
		(0.074)						
SES Index			0.005***	0.004***	0.004***	0.009**	0.004***	0.009**
(percentile)			(0.001)	(0.001)	(0.001)	(0.004)	(0.001)	(0.004)
(SES Index) ^{2}						-0.0001		-0.0001
						(0.0000)		(0.0000)
Mother's Educ $\geq$ 6y							0.142***	0.156***
							(0.054)	(0.054)
Locality Controls								
Marginality				-0.001	0.005*	0.005*	0.005*	0.005*
(percentile)				(0.002)	(0.003)	(0.003)	(0.003)	(0.003)
Children 6-17y (%)				-0.031*	-0.044*	-0.045**	-0.044*	-0.044*
				(0.017)	(0.023)	(0.023)	(0.023)	(0.023)
Physicians per 1000s				0.044	0.038	0.037	0.038	0.035
				(0.032)	(0.029)	(0.029)	(0.029)	(0.029)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	0.14	0.14	0.14	0.14	0.14	0.14	0.14	0.14
Observations	2,700	2,700	2,700	2,700	2,700	2,700	2,700	2,700
R^2	0.024	0.026	0.035	0.040	0.066	0.067	0.069	0.070

Notes: Sample restricted to birth cohorts from 1987 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.14: Intent-to-Treat Effects on Girls' Weight

	Girls' Weight (kg)							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.791**	0.772**	0.787**	0.804**	0.845***	0.789**	0.837***	0.773**
	(0.322)	(0.324)	(0.325)	(0.322)	(0.315)	(0.317)	(0.312)	(0.313)
Age		1.449**						
		(0.660)						
SES Index			0.037***	0.028**	0.025*	0.121***	0.021	0.128***
(percentile)			(0.014)	(0.014)	(0.013)	(0.036)	(0.014)	(0.035)
(SES Index) ^{2}						-0.0011***		-0.0012***
						(0.0004)		(0.0004)
Mother's Educ $\geq$ 6y							1.191	1.474*
							(0.797)	(0.773)
Locality Controls								
Marginality				-0.040**	-0.001	0.004	0.002	0.008
(percentile)				(0.018)	(0.024)	(0.025)	(0.024)	(0.024)
Children 6-17y (%)				-0.001	-0.349	-0.329	-0.350	-0.328
				(0.206)	(0.278)	(0.279)	(0.277)	(0.279)
Physicians per 1000s				0.475	0.503	0.560	0.494	0.556
				(0.333)	(0.379)	(0.382)	(0.371)	(0.372)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	42.8	42.8	42.8	42.8	42.8	42.8	42.8	42.8
Observations	2,929	2,929	2,929	2,929	2,929	2,929	2,929	2,929
R^2	0.335	0.337	0.340	0.344	0.358	0.361	0.359	0.363

Notes: Sample restricted to birth cohorts from 1987 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.15: Intent-to-Treat Effects on Girls' Standardized Weight for Age

	Girls' Weight-for-Age (z-score)							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.082**	0.082**	0.083**	0.085***	0.088***	0.083***	0.088***	0.082***
	(0.033)	(0.033)	(0.033)	(0.033)	(0.031)	(0.031)	(0.031)	(0.031)
Age		-0.007						
		(0.062)						
SES Index			0.004***	0.003**	0.003**	0.013***	0.003**	0.013***
(percentile)			(0.001)	(0.001)	(0.001)	(0.003)	(0.001)	(0.003)
(SES Index) ^{2}						-0.0001***		-0.0001***
						(0.0000)		(0.0000)
Mother's Educ $\geq$ 6y							0.057	0.085
							(0.069)	(0.068)
Locality Controls								
Marginality				-0.003**	0.001	0.002	0.001	0.002
(percentile)				(0.002)	(0.002)	(0.002)	(0.002)	(0.002)
Children 6-17y (%)				-0.027*	-0.064***	-0.062***	-0.064***	-0.062***
				(0.016)	(0.022)	(0.022)	(0.022)	(0.022)
Physicians per 1000s				0.032	0.037	0.043	0.037	0.042
				(0.033)	(0.036)	(0.036)	(0.036)	(0.036)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort × Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	0.13	0.13	0.13	0.13	0.13	0.13	0.13	0.13
Observations	2,910	2,910	2,910	2,910	2,910	2,910	2,910	2,910
R^2	0.027	0.027	0.036	0.046	0.064	0.069	0.065	0.070

Notes: Sample restricted to birth cohorts from 1987 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.16: Intent-to-Treat Effects on Boys' BMI

	Boys' BMI							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.102	0.097	0.101	0.100	0.131	0.128	0.130	0.126
	(0.093)	(0.092)	(0.093)	(0.092)	(0.095)	(0.093)	(0.096)	(0.093)
Age		0.235						
		(0.266)						
SES Index			0.005	0.004	0.005	0.026*	0.004	0.027*
(percentile)			(0.004)	(0.004)	(0.004)	(0.015)	(0.004)	(0.015)
(SES Index) ^{2}						-0.0002		-0.0003
						(0.0002)		(0.0002)
Mother's Educ $\geq$ 6y							0.090	0.147
							(0.180)	(0.180)
Locality Controls								
Marginality				0.001	0.007	0.007	0.008	0.008
(percentile)				(0.006)	(0.008)	(0.008)	(0.008)	(0.008)
Children 6-17y (%)				-0.064	-0.032	-0.033	-0.031	-0.032
				(0.054)	(0.074)	(0.075)	(0.074)	(0.075)
Physicians per 1000s				0.019	0.028	0.020	0.028	0.018
				(0.108)	(0.116)	(0.116)	(0.116)	(0.116)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	19.8	19.8	19.8	19.8	19.8	19.8	19.8	19.8
Observations	2,697	2,697	2,697	2,697	2,697	2,697	2,697	2,697
R^2	0.198	0.199	0.199	0.200	0.214	0.216	0.214	0.216

Notes: Sample restricted to birth cohorts from 1987 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.17: Intent-to-Treat Effects on Boys' Standardized BMI for Age

	Boys' BMI-for-Age (z-score)							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.010	0.012	0.010	0.010	0.021	0.020	0.021	0.020
	(0.027)	(0.027)	(0.027)	(0.026)	(0.026)	(0.025)	(0.026)	(0.025)
Age		-0.079						
		(0.070)						
SES Index			0.001	0.001	0.001	0.006	0.001	0.006
(percentile)			(0.001)	(0.001)	(0.001)	(0.005)	(0.001)	(0.005)
(SES Index)^2						-0.0001		-0.0001
						(0.0001)		(0.0001)
Mother's Educ $\geq$ 6y							0.026	0.038
							(0.053)	(0.052)
Locality Controls								
Marginality				0.002	0.003	0.003	0.003	0.003
(percentile)				(0.002)	(0.002)	(0.002)	(0.002)	(0.002)
Children 6-17y (%)				-0.028*	-0.011	-0.011	-0.011	-0.011
				(0.016)	(0.021)	(0.021)	(0.021)	(0.021)
Physicians per 1000s				0.009	0.010	0.008	0.010	0.007
				(0.036)	(0.039)	(0.039)	(0.039)	(0.039)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	0.47	0.47	0.47	0.47	0.47	0.47	0.47	0.47
Observations	2,697	2,697	2,697	2,697	2,697	2,697	2,697	2,697
R^2	0.013	0.014	0.014	0.015	0.034	0.036	0.034	0.036

Notes: Sample restricted to birth cohorts from 1987 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.18: Intent-to-Treat Effects on Girls' BMI

	Girls' BMI							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.368***	0.366***	0.368***	0.367***	0.390***	0.369***	0.391***	0.369***
	(0.124)	(0.125)	(0.125)	(0.124)	(0.120)	(0.120)	(0.120)	(0.121)
Age		0.242						
		(0.245)						
SES Index			0.006	0.005	0.005	0.043***	0.005	0.043***
(percentile)			(0.005)	(0.005)	(0.005)	(0.014)	(0.005)	(0.013)
(SES Index) ^{2}						-0.0004***		-0.0004***
						(0.0002)		(0.0002)
Mother's Educ $\geq$ 6y							-0.109	-0.004
							(0.307)	(0.300)
Locality Controls								
Marginality				0.002	0.014	0.016*	0.014	0.016*
(percentile)				(0.007)	(0.009)	(0.009)	(0.009)	(0.009)
Children 6-17y (%)				-0.080	-0.173*	-0.165	-0.173*	-0.165
				(0.070)	(0.100)	(0.102)	(0.100)	(0.102)
Physicians per 1000s				0.243**	0.274**	0.295**	0.274**	0.295**
				(0.105)	(0.118)	(0.116)	(0.118)	(0.116)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	20.0	20.0	20.0	20.0	20.0	20.0	20.0	20.0
Observations	2,907	2,907	2,907	2,907	2,907	2,907	2,907	2,907
R^2	0.191	0.192	0.192	0.195	0.209	0.213	0.209	0.213

Notes: Sample restricted to birth cohorts from 1987 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.19: Intent-to-Treat Effects on Girls' Standardized BMI for Age

	Girls' BMI-for-Age (z-score)							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.074**	0.074**	0.074**	0.073**	0.078***	0.073**	0.078***	0.073**
	(0.030)	(0.030)	(0.030)	(0.030)	(0.028)	(0.028)	(0.028)	(0.029)
Age		0.001						
		(0.057)						
SES Index			0.001	0.001	0.001	0.010***	0.001	0.010***
(percentile)			(0.001)	(0.001)	(0.001)	(0.003)	(0.001)	(0.003)
(SES Index) ^{2}						-0.0001***		-0.0001***
						(0.0000)		(0.0000)
Mother's Educ $\geq$ 6y							-0.053	-0.030
							(0.061)	(0.060)
Locality Controls								
Marginality				0.002	0.003*	0.004*	0.003	0.004*
(percentile)				(0.002)	(0.002)	(0.002)	(0.002)	(0.002)
Children 6-17y (%)				-0.034**	-0.050***	-0.048**	-0.050***	-0.048**
				(0.014)	(0.019)	(0.019)	(0.019)	(0.019)
Physicians per 1000s				0.045*	0.050*	0.055**	0.050*	0.055**
				(0.025)	(0.027)	(0.026)	(0.027)	(0.026)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort × Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	0.42	0.42	0.42	0.42	0.42	0.42	0.42	0.42
Observations	2,907	2,907	2,907	2,907	2,907	2,907	2,907	2,907
R^2	0.021	0.021	0.022	0.027	0.044	0.049	0.044	0.049

Notes: Sample restricted to birth cohorts from 1987 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.20: Intent-to-Treat Effects on Boys' Underweight Prevalence

	Boys' Underweight							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.0124	0.0127	0.0122	0.0122	0.0087	0.0089	0.0086	0.0090
	(0.0087)	(0.0088)	(0.0087)	(0.0086)	(0.0086)	(0.0086)	(0.0085)	(0.0085)
Age		-0.0148						
		(0.0167)						
SES Index			0.0002	0.0001	0.0002	-0.0021	0.0001	-0.0021
(percentile)			(0.0004)	(0.0004)	(0.0004)	(0.0016)	(0.0004)	(0.0016)
$(\text{SES Index})^2$						0.00003		0.00003
						(0.00002)		(0.00002)
Mother's Educ $\geq$ 6y							0.0049	-0.0011
							(0.0212)	(0.0193)
Locality Controls								
Marginality				-0.0004	-0.0002	-0.0002	-0.0002	-0.0002
(percentile)				(0.0006)	(0.0008)	(0.0007)	(0.0008)	(0.0007)
Children 6-17y (%)				0.0012	-0.0067	-0.0069	-0.0067	-0.0069
				(0.0053)	(0.0052)	(0.0051)	(0.0052)	(0.0051)
Physicians per 1000s				-0.0060	-0.0043	-0.0032	-0.0044	-0.0032
				(0.0142)	(0.0117)	(0.0114)	(0.0117)	(0.0115)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	0.066	0.066	0.066	0.066	0.066	0.066	0.066	0.066
Observations	1,792	1,792	1,792	1,792	1,792	1,792	1,792	1,792
R^2	0.023	0.023	0.023	0.024	0.049	0.053	0.049	0.053

Notes: Sample restricted to birth cohorts from 1987 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.21: Intent-to-Treat Effects on Girls' Underweight Prevalence

	Girls' Underweight							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	-0.0032	-0.0030	-0.0031	-0.0027	-0.0026	-0.0020	-0.0030	-0.0024
	(0.0080)	(0.0080)	(0.0079)	(0.0079)	(0.0078)	(0.0078)	(0.0079)	(0.0079)
Age		-0.0119						
		(0.0173)						
SES Index			0.0003	0.0002	0.0001	-0.0009	0.0000	-0.0008
(percentile)			(0.0003)	(0.0003)	(0.0003)	(0.0009)	(0.0003)	(0.0009)
(SES Index)^2						0.00001		0.00001
						(0.00001)		(0.00001)
Mother's Educ $\geq$ 6y							0.0195	0.0172
							(0.0157)	(0.0150)
Locality Controls								
Marginality				-0.0009**	-0.0000	-0.0001	0.0000	-0.0001
(percentile)				(0.0004)	(0.0006)	(0.0006)	(0.0006)	(0.0006)
Children 6-17y (%)				0.0070*	0.0003	0.0001	0.0003	0.0001
				(0.0039)	(0.0058)	(0.0058)	(0.0057)	(0.0058)
Physicians per 1000s				-0.0001	0.0038	0.0035	0.0040	0.0037
				(0.0066)	(0.0052)	(0.0052)	(0.0050)	(0.0050)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	0.067	0.067	0.067	0.067	0.067	0.067	0.067	0.067
Observations	2,022	2,022	2,022	2,022	2,022	2,022	2,022	2,022
R^2	0.005	0.006	0.007	0.011	0.029	0.030	0.030	0.031

Notes: Sample restricted to birth cohorts from 1987 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.22: Intent-to-Treat Effects on Boys' Overweight Prevalence

	Boys' Overweight							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.0041	0.0045	0.0040	0.0037	0.0024	0.0021	0.0026	0.0022
	(0.0123)	(0.0122)	(0.0124)	(0.0122)	(0.0127)	(0.0124)	(0.0127)	(0.0124)
Age		-0.0236						
		(0.0278)						
SES Index			0.0005	0.0006	0.0006	0.0029*	0.0007	0.0029*
(percentile)			(0.0005)	(0.0006)	(0.0005)	(0.0015)	(0.0005)	(0.0015)
(SES Index) ^{2}						-0.00003		-0.00003
						(0.00002)		(0.00002)
Mother's Educ $\geq$ 6y							-0.0180	-0.0131
							(0.0260)	(0.0256)
Locality Controls								
Marginality				0.0008	0.0014	0.0014	0.0014	0.0014
(percentile)				(0.0008)	(0.0011)	(0.0011)	(0.0011)	(0.0011)
Children 6-17y (%)				-0.0066	-0.0106	-0.0106	-0.0107	-0.0106
				(0.0064)	(0.0090)	(0.0090)	(0.0090)	(0.0090)
Physicians per 1000s				0.0056	0.0095	0.0088	0.0096	0.0090
				(0.0186)	(0.0154)	(0.0153)	(0.0154)	(0.0153)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	0.317	0.317	0.317	0.317	0.317	0.317	0.317	0.317
Observations	2,339	2,339	2,339	2,339	2,339	2,339	2,339	2,339
R^2	0.009	0.010	0.010	0.011	0.032	0.034	0.032	0.034

Notes: Sample restricted to birth cohorts from 1987 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.23: Intent-to-Treat Effects on Girls' Overweight Prevalence

	Girls' Overweight							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.0293**	0.0296**	0.0293**	0.0294**	0.0295**	0.0280**	0.0293**	0.0276*
	(0.0145)	(0.0146)	(0.0145)	(0.0145)	(0.0139)	(0.0142)	(0.0139)	(0.0142)
Age		-0.0300						
		(0.0260)						
SES Index			0.0005	0.0004	0.0005	0.0031**	0.0004	0.0033**
(percentile)			(0.0005)	(0.0005)	(0.0005)	(0.0015)	(0.0005)	(0.0015)
$(\text{SES Index})^2$						-0.00003*		-0.00003**
						(0.00002)		(0.00002)
Mother's Educ $\geq$ 6y							0.0171	0.0244
							(0.0262)	(0.0263)
Locality Controls								
Marginality				-0.0002	0.0012	0.0014	0.0013	0.0014
(percentile)				(0.0007)	(0.0009)	(0.0010)	(0.0009)	(0.0010)
Children 6-17y (%)				-0.0021	-0.0113	-0.0104	-0.0113	-0.0103
				(0.0064)	(0.0076)	(0.0076)	(0.0076)	(0.0076)
Physicians per 1000s				0.0104	0.0183*	0.0197**	0.0181*	0.0196*
				(0.0097)	(0.0099)	(0.0099)	(0.0100)	(0.0100)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	0.285	0.285	0.285	0.285	0.285	0.285	0.285	0.285
Observations	2,647	2,647	2,647	2,647	2,647	2,647	2,647	2,647
R^2	0.021	0.021	0.022	0.022	0.038	0.040	0.038	0.041

Notes: Sample restricted to birth cohorts from 1987 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.24: Intent-to-Treat Effects on Boys' Obesity Prevalence

	Boys' Obesity							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.0096	0.0096	0.0091	0.0090	0.0106*	0.0104*	0.0100	0.0097
	(0.0063)	(0.0063)	(0.0063)	(0.0063)	(0.0063)	(0.0063)	(0.0064)	(0.0064)
Age		-0.0034						
		(0.0181)						
SES Index			0.0006**	0.0005*	0.0005**	0.0016*	0.0004*	0.0016*
(percentile)			(0.0002)	(0.0003)	(0.0003)	(0.0009)	(0.0003)	(0.0009)
(SES Index) ^{2}						-0.00001		-0.00001
						(0.00001)		(0.00001)
Mother's Educ $\geq$ 6y							0.0215	0.0238
							(0.0161)	(0.0165)
Locality Controls								
Marginality				-0.0008*	-0.0001	-0.0001	-0.0001	-0.0001
(percentile)				(0.0004)	(0.0007)	(0.0007)	(0.0007)	(0.0007)
Children 6-17y (%)				0.0027	0.0006	0.0006	0.0006	0.0006
				(0.0041)	(0.0059)	(0.0060)	(0.0059)	(0.0060)
Physicians per 1000s				0.0006	0.0001	-0.0002	-0.0003	-0.0007
				(0.0077)	(0.0078)	(0.0079)	(0.0077)	(0.0078)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	0.115	0.115	0.115	0.115	0.115	0.115	0.115	0.115
Observations	1,872	1,872	1,872	1,872	1,872	1,872	1,872	1,872
R^2	0.014	0.014	0.017	0.020	0.037	0.038	0.038	0.039

Notes: Sample restricted to birth cohorts from 1987 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.25: Intent-to-Treat Effects on Girls' Obesity Prevalence

	Girls' Obesity							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.0167***	0.0171***	0.0168***	0.0171***	0.0177***	0.0167***	0.0178***	0.0169***
	(0.0055)	(0.0056)	(0.0056)	(0.0055)	(0.0058)	(0.0057)	(0.0058)	(0.0057)
Age		-0.0220						
		(0.0176)						
SES Index			0.0006**	0.0005**	0.0006**	0.0020**	0.0006**	0.0020**
(percentile)			(0.0003)	(0.0003)	(0.0003)	(0.0008)	(0.0003)	(0.0008)
$(\text{SES Index})^2$						-0.00002*		-0.00002*
						(0.00001)		(0.00001)
Mother's Educ $\geq$ 6y							-0.0097	-0.0061
							(0.0172)	(0.0168)
Locality Controls								
Marginality				-0.0003	0.0003	0.0004	0.0002	0.0004
(percentile)				(0.0004)	(0.0005)	(0.0005)	(0.0005)	(0.0005)
Children 6-17y (%)				0.0005	-0.0029	-0.0026	-0.0029	-0.0026
				(0.0048)	(0.0067)	(0.0067)	(0.0067)	(0.0067)
Physicians per 1000s				-0.0009	-0.0006	-0.0003	-0.0007	-0.0004
				(0.0059)	(0.0060)	(0.0061)	(0.0060)	(0.0060)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	0.072	0.072	0.072	0.072	0.072	0.072	0.072	0.072
Observations	2,026	2,026	2,026	2,026	2,026	2,026	2,026	2,026
R^2	0.022	0.024	0.027	0.028	0.045	0.048	0.046	0.048

Notes: Sample restricted to birth cohorts from 1987 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.26: Intent-to-Treat Effects on Boys' Height

	Boys' Height (cm)							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.639**	0.596*	0.735**	0.715**	0.878***	0.875***	0.881***	0.875***
	(0.303)	(0.304)	(0.296)	(0.289)	(0.256)	(0.254)	(0.258)	(0.256)
Age		1.466***						
		(0.429)						
SES Index			0.053***	0.046***	0.046***	0.052***	0.042***	0.055***
(percentile)			(0.006)	(0.006)	(0.005)	(0.020)	(0.006)	(0.019)
$(\text{SES Index})^2$						-0.0001		-0.0001
						(0.0002)		(0.0002)
Mother's Educ $\geq$ 6y							0.826*	0.857*
							(0.466)	(0.471)
Locality Controls								
Marginality				-0.049***	0.001	0.001	0.003	0.003
(percentile)				(0.012)	(0.014)	(0.014)	(0.013)	(0.013)
Children 6-17y (%)				0.127	-0.265**	-0.266**	-0.270**	-0.272**
				(0.118)	(0.118)	(0.118)	(0.116)	(0.116)
Physicians per 1000s				0.114	0.062	0.063	0.041	0.044
				(0.221)	(0.195)	(0.195)	(0.191)	(0.192)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	154.0	154.0	154.0	154.0	154.0	154.0	154.0	154.0
Observations	5,254	5,254	5,254	5,254	5,254	5,254	5,254	5,254
R^2	0.647	0.649	0.660	0.665	0.678	0.678	0.679	0.679

Notes: Sample restricted to birth cohorts from 1983 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.27: Intent-to-Treat Effects on Boys' Standardized Height for Age

	Boys' Height-for-Age (z-score)							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.067	0.069	0.081*	0.078*	0.099***	0.100***	0.099***	0.098***
	(0.043)	(0.043)	(0.042)	(0.040)	(0.035)	(0.034)	(0.034)	(0.034)
Age		-0.041						
		(0.055)						
SES Index			0.007***	0.006***	0.007***	0.006**	0.006***	0.007**
(percentile)			(0.001)	(0.001)	(0.001)	(0.003)	(0.001)	(0.003)
(SES Index) ^{2}						0.0000		-0.0000
						(0.0000)		(0.0000)
Mother's Educ $\geq$ 6y							0.137**	0.139**
							(0.056)	(0.056)
Locality Controls								
Marginality				-0.005***	0.002	0.002	0.002	0.002
(percentile)				(0.002)	(0.002)	(0.002)	(0.002)	(0.002)
Children 6-17y (%)				0.011	-0.047***	-0.047***	-0.047***	-0.047***
				(0.016)	(0.016)	(0.016)	(0.016)	(0.016)
Physicians per 1000s				0.041	0.029	0.029	0.026	0.026
				(0.031)	(0.023)	(0.023)	(0.023)	(0.023)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	-0.49	-0.49	-0.49	-0.49	-0.49	-0.49	-0.49	-0.49
Observations	4,644	4,644	4,644	4,644	4,644	4,644	4,644	4,644
R^2	0.075	0.076	0.112	0.122	0.157	0.157	0.161	0.161

Notes: Sample restricted to birth cohorts from 1983 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.28: Intent-to-Treat Effects on Girls' Height

	Girls' Height (cm)							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.068	0.059	0.081	0.129	0.215	0.216	0.247	0.237
	(0.269)	(0.267)	(0.262)	(0.250)	(0.243)	(0.242)	(0.243)	(0.241)
Age		0.469						
		(0.435)						
SES Index			0.036***	0.025***	0.024***	0.023	0.019***	0.029*
(percentile)			(0.005)	(0.005)	(0.005)	(0.016)	(0.005)	(0.015)
(SES Index) ^{2}						0.0000		-0.0001
						(0.0002)		(0.0002)
Mother's Educ $\geq$ 6y							1.266***	1.294***
							(0.253)	(0.255)
Locality Controls								
Marginality				-0.070***	-0.026**	-0.026**	-0.023**	-0.023*
(percentile)				(0.010)	(0.012)	(0.012)	(0.012)	(0.012)
Children 6-17y (%)				0.158	-0.151	-0.151	-0.152	-0.151
				(0.099)	(0.109)	(0.109)	(0.108)	(0.107)
Physicians per 1000s				-0.256	-0.237	-0.237	-0.242	-0.242
				(0.202)	(0.221)	(0.221)	(0.217)	(0.217)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	150.9	150.9	150.9	150.9	150.9	150.9	150.9	150.9
Observations	5,913	5,913	5,913	5,913	5,913	5,913	5,913	5,913
R^2	0.328	0.328	0.342	0.362	0.388	0.388	0.393	0.393

Notes: Sample restricted to birth cohorts from 1983 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.29: Intent-to-Treat Effects on Girls' Standardized Height for Age

	Girls' Height-for-Age (z-score)							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.020	0.023	0.023	0.023	0.033	0.033	0.036	0.034
	(0.047)	(0.047)	(0.046)	(0.044)	(0.042)	(0.042)	(0.042)	(0.041)
Age		-0.093						
		(0.070)						
SES Index			0.005***	0.004***	0.004***	0.004	0.003***	0.005*
(percentile)			(0.001)	(0.001)	(0.001)	(0.003)	(0.001)	(0.003)
(SES Index) ^{2}						-0.0000		-0.0000
						(0.0000)		(0.0000)
Mother's Educ $\geq$ 6y							0.183***	0.188***
							(0.046)	(0.046)
Locality Controls								
Marginality				-0.010***	-0.003*	-0.003*	-0.003	-0.002
(percentile)				(0.001)	(0.002)	(0.002)	(0.002)	(0.002)
Children 6-17y (%)				0.023*	-0.024	-0.024	-0.023	-0.023
				(0.014)	(0.016)	(0.016)	(0.016)	(0.016)
Physicians per 1000s				-0.039	-0.042	-0.042	-0.045	-0.044
				(0.030)	(0.034)	(0.034)	(0.033)	(0.033)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort × Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	-0.61	-0.61	-0.61	-0.61	-0.61	-0.61	-0.61	-0.61
Observations	5,190	5,190	5,190	5,190	5,190	5,190	5,190	5,190
R^2	0.084	0.085	0.102	0.125	0.156	0.156	0.162	0.162

Notes: Sample restricted to birth cohorts from 1983 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.30: Intent-to-Treat Effects on Boys' Weight

	Boys' Weight (kg)							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.970**	0.915**	1.055**	1.063**	1.253***	1.246***	1.258***	1.245***
	(0.457)	(0.442)	(0.438)	(0.439)	(0.447)	(0.447)	(0.442)	(0.441)
Age		2.103***						
		(0.619)						
SES Index			0.047***	0.041***	0.044***	0.057*	0.039***	0.061*
(percentile)			(0.008)	(0.008)	(0.008)	(0.033)	(0.008)	(0.032)
(SES Index) ^{2}						-0.0002		-0.0002
						(0.0004)		(0.0004)
Mother's Educ $\geq$ 6y							1.054**	1.109**
							(0.515)	(0.500)
Locality Controls								
Marginality				-0.028*	0.021	0.021	0.024	0.024
(percentile)				(0.015)	(0.019)	(0.019)	(0.019)	(0.019)
Children 6-17y (%)				-0.195	-0.386*	-0.388*	-0.391*	-0.395*
				(0.144)	(0.204)	(0.205)	(0.204)	(0.204)
Physicians per 1000s				0.122	0.064	0.067	0.042	0.046
				(0.334)	(0.309)	(0.308)	(0.304)	(0.303)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort × Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	50.3	50.3	50.3	50.3	50.3	50.3	50.3	50.3
Observations	5,298	5,298	5,298	5,298	5,298	5,298	5,298	5,298
R^2	0.444	0.446	0.449	0.451	0.462	0.462	0.462	0.462

Notes: Sample restricted to birth cohorts from 1983 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.31: Intent-to-Treat Effects on Boys' Standardized Weight for Age

	Boys' Weight-for-Age (z-score)							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.079	0.078	0.089*	0.090*	0.109**	0.106**	0.108**	0.105**
	(0.053)	(0.052)	(0.050)	(0.049)	(0.049)	(0.048)	(0.048)	(0.047)
Age		0.026						
		(0.062)						
SES Index			0.005***	0.005***	0.005***	0.010***	0.004***	0.010***
(percentile)			(0.001)	(0.001)	(0.001)	(0.004)	(0.001)	(0.003)
(SES Index) ^{2}						-0.0001		-0.0001*
						(0.0000)		(0.0000)
Mother's Educ $\geq$ 6y							0.146***	0.160***
							(0.045)	(0.044)
Locality Controls								
Marginality				-0.002	0.002	0.002	0.003	0.003
(percentile)				(0.002)	(0.002)	(0.002)	(0.002)	(0.002)
Children 6-17y (%)				-0.023	-0.039**	-0.039**	-0.040**	-0.040**
				(0.014)	(0.019)	(0.019)	(0.019)	(0.019)
Physicians per 1000s				0.036	0.032	0.034	0.029	0.030
				(0.030)	(0.025)	(0.025)	(0.025)	(0.025)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	0.08	0.08	0.08	0.08	0.08	0.08	0.08	0.08
Observations	4,637	4,637	4,637	4,637	4,637	4,637	4,637	4,637
R^2	0.020	0.020	0.034	0.040	0.058	0.059	0.061	0.063

Notes: Sample restricted to birth cohorts from 1983 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.32: Intent-to-Treat Effects on Girls' Weight

	Girls' Weight (kg)							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.212	0.190	0.224	0.261	0.345	0.279	0.360	0.291
	(0.484)	(0.481)	(0.483)	(0.476)	(0.471)	(0.477)	(0.469)	(0.474)
Age		0.829						
		(0.637)						
SES Index			0.028***	0.021**	0.020**	0.083***	0.017*	0.086***
(percentile)			(0.009)	(0.009)	(0.009)	(0.028)	(0.009)	(0.028)
(SES Index) ^{2}						-0.0007**		-0.0008**
						(0.0003)		(0.0003)
Mother's Educ $\geq$ 6y							0.653	0.839
							(0.576)	(0.572)
Locality Controls								
Marginality				-0.042***	-0.009	-0.006	-0.007	-0.004
(percentile)				(0.015)	(0.015)	(0.015)	(0.015)	(0.015)
Children 6-17y (%)				0.004	-0.264*	-0.263*	-0.264*	-0.263*
				(0.161)	(0.158)	(0.157)	(0.157)	(0.156)
Physicians per 1000s				-0.142	-0.169	-0.170	-0.171	-0.173
				(0.213)	(0.233)	(0.232)	(0.234)	(0.233)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	49.5	49.5	49.5	49.5	49.5	49.5	49.5	49.5
Observations	5,823	5,823	5,823	5,823	5,823	5,823	5,823	5,823
R^2	0.248	0.248	0.251	0.254	0.267	0.268	0.268	0.269

Notes: Sample restricted to birth cohorts from 1983 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.33: Intent-to-Treat Effects on Girls' Standardized Weight for Age

	Girls' Weight-for-Age (z-score)							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.036	0.038	0.038	0.039	0.042	0.034	0.042	0.034
	(0.042)	(0.043)	(0.042)	(0.041)	(0.039)	(0.038)	(0.039)	(0.038)
Age		-0.063						
		(0.043)						
SES Index			0.003***	0.002***	0.002***	0.009***	0.002***	0.009***
(percentile)			(0.001)	(0.001)	(0.001)	(0.002)	(0.001)	(0.003)
(SES Index) ^{2}						-0.0001***		-0.0001***
						(0.0000)		(0.0000)
Mother's Educ $\geq$ 6y							-0.020	-0.003
							(0.055)	(0.056)
Locality Controls								
Marginality				-0.004***	-0.001	-0.001	-0.001	-0.001
(percentile)				(0.001)	(0.002)	(0.002)	(0.002)	(0.002)
Children 6-17y (%)				-0.012	-0.034**	-0.034**	-0.034**	-0.034**
				(0.014)	(0.017)	(0.017)	(0.017)	(0.017)
Physicians per 1000s				0.008	0.015	0.017	0.015	0.017
				(0.028)	(0.029)	(0.029)	(0.029)	(0.029)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort × Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	0.14	0.14	0.14	0.14	0.14	0.14	0.14	0.14
Observations	5,084	5,084	5,084	5,084	5,084	5,084	5,084	5,084
R^2	0.019	0.020	0.025	0.033	0.051	0.053	0.051	0.053

Notes: Sample restricted to birth cohorts from 1983 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.34: Intent-to-Treat Effects on Boys' BMI

	Boys' BMI							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.169	0.155	0.178	0.190	0.227	0.222	0.228	0.222
	(0.174)	(0.170)	(0.171)	(0.170)	(0.176)	(0.175)	(0.175)	(0.174)
Age		0.472**						
		(0.198)						
SES Index			0.005*	0.005	0.005*	0.015	0.004	0.015
(percentile)			(0.003)	(0.003)	(0.003)	(0.011)	(0.003)	(0.011)
(SES Index) ^{2}						-0.0001		-0.0001
						(0.0001)		(0.0001)
Mother's Educ $\geq$ 6y							0.156	0.183
							(0.171)	(0.165)
Locality Controls								
Marginality				0.003	0.010	0.010	0.010	0.010
(percentile)				(0.005)	(0.007)	(0.007)	(0.007)	(0.007)
Children 6-17y (%)				-0.117**	-0.103	-0.105	-0.104	-0.106
				(0.057)	(0.084)	(0.084)	(0.084)	(0.084)
Physicians per 1000s				0.068	0.065	0.068	0.062	0.063
				(0.105)	(0.106)	(0.105)	(0.106)	(0.104)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort × Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	20.8	20.8	20.8	20.8	20.8	20.8	20.8	20.8
Observations	5,242	5,242	5,242	5,242	5,242	5,242	5,242	5,242
R^2	0.197	0.199	0.198	0.200	0.209	0.210	0.210	0.210

Notes: Sample restricted to birth cohorts from 1983 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.35: Intent-to-Treat Effects on Boys' Standardized BMI for Age

	Boys' BMI-for-Age (z-score)							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.041	0.040	0.044	0.048	0.057	0.055	0.057	0.054
	(0.055)	(0.054)	(0.054)	(0.052)	(0.055)	(0.054)	(0.054)	(0.053)
Age		0.045						
		(0.060)						
SES Index			0.001	0.001	0.002*	0.006*	0.001	0.007*
(percentile)			(0.001)	(0.001)	(0.001)	(0.003)	(0.001)	(0.003)
(SES Index)^2						-0.0001		-0.0001
						(0.0000)		(0.0000)
Mother's Educ $\geq$ 6y							0.060	0.072
							(0.046)	(0.044)
Locality Controls								
Marginality				0.002	0.002	0.002	0.002	0.002
(percentile)				(0.001)	(0.002)	(0.002)	(0.002)	(0.002)
Children 6-17y (%)				-0.035**	-0.015	-0.015	-0.016	-0.016
				(0.016)	(0.020)	(0.020)	(0.020)	(0.020)
Physicians per 1000s				0.012	0.014	0.015	0.013	0.014
				(0.028)	(0.025)	(0.025)	(0.025)	(0.025)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	0.39	0.39	0.39	0.39	0.39	0.39	0.39	0.39
Observations	4,632	4,632	4,632	4,632	4,632	4,632	4,632	4,632
R^2	0.011	0.012	0.012	0.015	0.028	0.029	0.029	0.030

Notes: Sample restricted to birth cohorts from 1983 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.36: Intent-to-Treat Effects on Girls' BMI

	Girls' BMI							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.120	0.117	0.121	0.122	0.134	0.108	0.130	0.106
	(0.179)	(0.179)	(0.179)	(0.178)	(0.178)	(0.180)	(0.179)	(0.181)
Age		0.162						
		(0.223)						
SES Index			0.001	0.001	0.001	0.027**	0.001	0.026**
(percentile)			(0.003)	(0.003)	(0.003)	(0.010)	(0.003)	(0.010)
(SES Index) ^{2}						-0.0003**		-0.0003**
						(0.0001)		(0.0001)
Mother's Educ $\geq$ 6y							-0.163	-0.094
							(0.207)	(0.207)
Locality Controls								
Marginality				0.001	0.003	0.004	0.003	0.004
(percentile)				(0.006)	(0.007)	(0.007)	(0.007)	(0.007)
Children 6-17y (%)				-0.036	-0.056	-0.056	-0.056	-0.056
				(0.057)	(0.067)	(0.067)	(0.068)	(0.067)
Physicians per 1000s				0.039	0.025	0.024	0.025	0.024
				(0.100)	(0.124)	(0.123)	(0.123)	(0.123)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort × Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	21.5	21.5	21.5	21.5	21.5	21.5	21.5	21.5
Observations	5,760	5,760	5,760	5,760	5,760	5,760	5,760	5,760
R^2	0.149	0.150	0.149	0.150	0.158	0.160	0.159	0.160

Notes: Sample restricted to birth cohorts from 1983 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.37: Intent-to-Treat Effects on Girls' Standardized BMI for Age

	Girls' BMI-for-Age (z-score)							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.036	0.037	0.037	0.037	0.035	0.027	0.033	0.026
	(0.044)	(0.044)	(0.044)	(0.043)	(0.040)	(0.040)	(0.040)	(0.040)
Age		-0.026						
		(0.044)						
SES Index			0.001	0.001	0.001	0.008***	0.001	0.007***
(percentile)			(0.001)	(0.001)	(0.001)	(0.002)	(0.001)	(0.002)
(SES Index) ^{2}						-0.0001***		-0.0001***
						(0.0000)		(0.0000)
Mother's Educ $\geq$ 6y							-0.107**	-0.092*
							(0.050)	(0.051)
Locality Controls								
Marginality				0.001	-0.000	0.000	-0.000	-0.000
(percentile)				(0.001)	(0.002)	(0.002)	(0.002)	(0.002)
Children 6-17y (%)				-0.024*	-0.016	-0.016	-0.017	-0.017
				(0.013)	(0.015)	(0.015)	(0.015)	(0.015)
Physicians per 1000s				0.023	0.033	0.035	0.034	0.036
				(0.028)	(0.029)	(0.028)	(0.029)	(0.028)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	0.49	0.49	0.49	0.49	0.49	0.49	0.49	0.49
Observations	5,081	5,081	5,081	5,081	5,081	5,081	5,081	5,081
R^2	0.013	0.013	0.013	0.016	0.032	0.036	0.035	0.037

Notes: Sample restricted to birth cohorts from 1983 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.38: Intent-to-Treat Effects on Boys' Underweight Prevalence

	Boys' Underweight							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	-0.0009	-0.0002	-0.0005	-0.0008	-0.0011	-0.0006	-0.0011	-0.0006
	(0.0129)	(0.0128)	(0.0129)	(0.0128)	(0.0129)	(0.0127)	(0.0129)	(0.0126)
Age		-0.0226						
		(0.0174)						
SES Index			0.0002	0.0001	0.0001	-0.0009	0.0001	-0.0009
(percentile)			(0.0003)	(0.0003)	(0.0003)	(0.0010)	(0.0003)	(0.0010)
(SES Index) ^{2}						0.00001		0.00001
						(0.00001)		(0.00001)
Mother's Educ $\geq$ 6y							0.0091	0.0068
							(0.0142)	(0.0136)
Locality Controls								
Marginality				-0.0005	-0.0001	-0.0001	-0.0001	-0.0001
(percentile)				(0.0003)	(0.0005)	(0.0005)	(0.0005)	(0.0005)
Children 6-17y (%)				-0.0012	-0.0087**	-0.0087**	-0.0088**	-0.0087**
				(0.0036)	(0.0038)	(0.0038)	(0.0038)	(0.0037)
Physicians per 1000s				-0.0045	-0.0000	-0.0002	-0.0002	-0.0003
				(0.0093)	(0.0095)	(0.0094)	(0.0095)	(0.0094)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	0.064	0.064	0.064	0.064	0.064	0.064	0.064	0.064
Observations	3,517	3,517	3,517	3,517	3,517	3,517	3,517	3,517
R^2	0.013	0.014	0.013	0.015	0.032	0.032	0.032	0.033

Notes: Sample restricted to birth cohorts from 1983 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.39: Intent-to-Treat Effects on Girls' Underweight Prevalence

	Girls' Underweight							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.0062	0.0062	0.0062	0.0067	0.0099	0.0105	0.0100	0.0105
	(0.0125)	(0.0125)	(0.0125)	(0.0125)	(0.0119)	(0.0118)	(0.0118)	(0.0118)
Age		0.0014						
		(0.0129)						
SES Index			0.0001	0.0001	0.0001	-0.0006	-0.0000	-0.0005
(percentile)			(0.0003)	(0.0003)	(0.0002)	(0.0007)	(0.0003)	(0.0007)
$(\text{SES Index})^2$						0.00001		0.00001
						(0.00001)		(0.00001)
Mother's Educ $\geq$ 6y							0.0215	0.0202
							(0.0151)	(0.0151)
Locality Controls								
Marginality				-0.0002	0.0009	0.0009	0.0009	0.0009
(percentile)				(0.0004)	(0.0007)	(0.0007)	(0.0007)	(0.0007)
Children 6-17y (%)				0.0079**	-0.0019	-0.0018	-0.0017	-0.0017
				(0.0039)	(0.0049)	(0.0049)	(0.0049)	(0.0049)
Physicians per 1000s				0.0051	0.0054	0.0053	0.0053	0.0053
				(0.0066)	(0.0065)	(0.0065)	(0.0065)	(0.0065)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	0.055	0.055	0.055	0.055	0.055	0.055	0.055	0.055
Observations	3,886	3,886	3,886	3,886	3,886	3,886	3,886	3,886
R^2	0.019	0.019	0.019	0.022	0.039	0.039	0.040	0.041

Notes: Sample restricted to birth cohorts from 1983 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.40: Intent-to-Treat Effects on Boys' Overweight Prevalence

	Boys' Overweight							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.0210	0.0207	0.0220	0.0223	0.0240	0.0240	0.0241	0.0240
	(0.0183)	(0.0181)	(0.0181)	(0.0176)	(0.0184)	(0.0184)	(0.0183)	(0.0183)
Age		0.0127						
		(0.0234)						
SES Index			0.0006*	0.0005	0.0006*	0.0006	0.0006	0.0006
(percentile)			(0.0003)	(0.0004)	(0.0004)	(0.0013)	(0.0004)	(0.0013)
(SES Index)^2						-0.00000		-0.00000
						(0.00001)		(0.00001)
Mother's Educ $\geq$ 6y							0.0069	0.0070
							(0.0203)	(0.0197)
Locality Controls								
Marginality				0.0002	0.0007	0.0007	0.0007	0.0007
(percentile)				(0.0006)	(0.0008)	(0.0008)	(0.0008)	(0.0008)
Children 6-17y (%)				-0.0131*	-0.0133	-0.0133	-0.0133	-0.0133
				(0.0070)	(0.0099)	(0.0098)	(0.0099)	(0.0098)
Physicians per 1000s				-0.0046	-0.0023	-0.0023	-0.0025	-0.0025
				(0.0111)	(0.0120)	(0.0120)	(0.0120)	(0.0120)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	0.330	0.330	0.330	0.330	0.330	0.330	0.330	0.330
Observations	4,977	4,977	4,977	4,977	4,977	4,977	4,977	4,977
R^2	0.035	0.035	0.036	0.039	0.047	0.047	0.047	0.047

Notes: Sample restricted to birth cohorts from 1983 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.41: Intent-to-Treat Effects on Girls' Overweight Prevalence

	Girls' Overweight							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.0378**	0.0380**	0.0379**	0.0378**	0.0392**	0.0367*	0.0393**	0.0369*
	(0.0185)	(0.0186)	(0.0185)	(0.0188)	(0.0188)	(0.0190)	(0.0189)	(0.0190)
Age		-0.0090						
		(0.0222)						
SES Index			0.0001	0.0001	0.0001	0.0026**	0.0001	0.0027**
(percentile)			(0.0003)	(0.0003)	(0.0003)	(0.0011)	(0.0003)	(0.0011)
$(\text{SES Index})^2$						-0.00003**		-0.00003**
						(0.00001)		(0.00001)
Mother's Educ $\geq$ 6y							0.0026	0.0094
							(0.0203)	(0.0206)
Locality Controls								
Marginality				-0.0004	0.0006	0.0007	0.0006	0.0007
(percentile)				(0.0006)	(0.0007)	(0.0007)	(0.0007)	(0.0007)
Children 6-17y (%)				0.0066	0.0011	0.0011	0.0011	0.0011
				(0.0059)	(0.0063)	(0.0063)	(0.0063)	(0.0063)
Physicians per 1000s				-0.0074	-0.0071	-0.0070	-0.0071	-0.0071
				(0.0094)	(0.0113)	(0.0113)	(0.0113)	(0.0113)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	0.341	0.341	0.341	0.341	0.341	0.341	0.341	0.341
Observations	5,520	5,520	5,520	5,520	5,520	5,520	5,520	5,520
R^2	0.024	0.024	0.024	0.025	0.039	0.041	0.039	0.041

Notes: Sample restricted to birth cohorts from 1983 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.42: Intent-to-Treat Effects on Boys' Obesity Prevalence

	Boys' Obesity							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.0120	0.0120	0.0123	0.0126	0.0156	0.0157	0.0157	0.0157
	(0.0102)	(0.0102)	(0.0101)	(0.0101)	(0.0106)	(0.0107)	(0.0104)	(0.0105)
Age		-0.0019						
		(0.0127)						
SES Index			0.0003	0.0002	0.0002	-0.0000	0.0001	0.0000
(percentile)			(0.0002)	(0.0002)	(0.0002)	(0.0008)	(0.0002)	(0.0008)
(SES Index) ^{2}						0.00000		0.00000
						(0.00001)		(0.00001)
Mother's Educ $\geq$ 6y							0.0210	0.0208
							(0.0137)	(0.0139)
Locality Controls								
Marginality				-0.0003	0.0004	0.0004	0.0005	0.0005
(percentile)				(0.0004)	(0.0005)	(0.0005)	(0.0005)	(0.0005)
Children 6-17y (%)				-0.0081**	-0.0092	-0.0092	-0.0094	-0.0094
				(0.0038)	(0.0058)	(0.0058)	(0.0058)	(0.0058)
Physicians per 1000s				0.0085	0.0094	0.0094	0.0089	0.0089
				(0.0083)	(0.0090)	(0.0090)	(0.0090)	(0.0090)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	0.095	0.095	0.095	0.095	0.095	0.095	0.095	0.095
Observations	3,679	3,679	3,679	3,679	3,679	3,679	3,679	3,679
R^2	0.041	0.041	0.042	0.048	0.058	0.058	0.059	0.059

Notes: Sample restricted to birth cohorts from 1983 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.43: Intent-to-Treat Effects on Girls' Obesity Prevalence

	Girls' Obesity							
	(1)	(2)	(3)	(4)	(5)	(6)	(7)	(8)
Years Treated	0.0203	0.0204	0.0201	0.0202	0.0226	0.0219	0.0226	0.0219
	(0.0139)	(0.0136)	(0.0138)	(0.0141)	(0.0143)	(0.0144)	(0.0143)	(0.0144)
Age		-0.0056						
		(0.0221)						
SES Index			0.0003	0.0004	0.0003	0.0012*	0.0003	0.0012*
(percentile)			(0.0002)	(0.0002)	(0.0002)	(0.0007)	(0.0002)	(0.0007)
(SES Index) ^{2}						-0.00001		-0.00001
						(0.00001)		(0.00001)
Mother's Educ $\geq$ 6y							-0.0057	-0.0036
							(0.0167)	(0.0169)
Locality Controls								
Marginality				0.0001	0.0009**	0.0010**	0.0009**	0.0010**
(percentile)				(0.0004)	(0.0005)	(0.0005)	(0.0004)	(0.0005)
Children 6-17y (%)				-0.0004	-0.0070	-0.0070	-0.0070	-0.0070
				(0.0041)	(0.0055)	(0.0056)	(0.0055)	(0.0056)
Physicians per 1000s				-0.0103	-0.0140	-0.0140	-0.0140	-0.0140
				(0.0092)	(0.0089)	(0.0088)	(0.0089)	(0.0088)
State FE	no	no	no	no	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes	yes
Mean DV	0.070	0.070	0.070	0.070	0.070	0.070	0.070	0.070
Observations	3,964	3,964	3,964	3,964	3,964	3,964	3,964	3,964
R^2	0.079	0.079	0.080	0.080	0.091	0.092	0.092	0.092

Notes: Sample restricted to birth cohorts from 1983 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.44: Intent-to-Treat Effects by Socioeconomic Status on Anthropometric Measures

	Height (cm)		Weight (kg)		BMI	
	(1) Boys	(2) Girls	(3) Boys	(4) Girls	(5) Boys	(6) Girls
Low SES × Years Treated	0.694***	0.349*	1.107***	1.269***	0.281**	0.534***
	(0.244)	(0.198)	(0.378)	(0.373)	(0.117)	(0.142)
High SES × Years Treated	0.208	0.207	0.146	0.673*	-0.037	0.322**
	(0.191)	(0.177)	(0.323)	(0.358)	(0.112)	(0.135)
SES Index	0.119***	0.107***	0.184***	0.221***	0.053***	0.060***
(percentile)	(0.035)	(0.026)	(0.042)	(0.043)	(0.017)	(0.017)
$(\text{SES Index})^2$	-0.0005	-0.0007***	-0.0012***	-0.0018***	-0.0004**	-0.0005***
	(0.0004)	(0.0003)	(0.0004)	(0.0005)	(0.0002)	(0.0002)
Locality Controls						
Marginality	0.042**	-0.029*	0.038	0.016	0.008	0.016
(percentile)	(0.016)	(0.015)	(0.026)	(0.026)	(0.009)	(0.010)
Children 6-17y (%)	-0.394**	-0.138	-0.205	-0.343	-0.023	-0.152
	(0.155)	(0.152)	(0.224)	(0.293)	(0.079)	(0.109)
Physicians per 1000s	0.489	-0.122	0.261	0.678*	0.006	0.339***
	(0.388)	(0.289)	(0.336)	(0.393)	(0.114)	(0.124)
State FE	yes	yes	yes	yes	yes	yes
Cohort × Time FE	yes	yes	yes	yes	yes	yes
Mean DV	144.1	145.2	41.7	42.8	19.8	20.0
Observations	2,511	2,720	2,530	2,688	2,506	2,670
R^2	0.771	0.502	0.561	0.366	0.227	0.211

Notes: Sample restricted to birth cohorts from 1987 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.45: Intent-to-Treat Effects by Socioeconomic Status on BMI Categories

	Underweight		Overweight		Obesity	
	(1) Boys	(2) Girls	(3) Boys	(4) Girls	(5) Boys	(6) Girls
Low SES × Years Treated	0.0116	-0.0110	0.0273*	0.0448***	0.0198**	0.0227***
	(0.0112)	(0.0078)	(0.0162)	(0.0158)	(0.0095)	(0.0072)
High SES × Years Treated	0.0096	0.0006	-0.0165	0.0188	0.0050	0.0131*
	(0.0103)	(0.0097)	(0.0138)	(0.0154)	(0.0074)	(0.0072)
SES Index	-0.0036*	-0.0010	0.0055***	0.0060***	0.0027**	0.0029***
(percentile)	(0.0021)	(0.0009)	(0.0018)	(0.0020)	(0.0011)	(0.0009)
(SES Index)^2	0.00004	0.00001	-0.00004**	-0.00005***	-0.00002*	-0.00002**
	(0.00002)	(0.00001)	(0.00002)	(0.00002)	(0.00001)	(0.00001)
Locality Controls						
Marginality	-0.0003	0.0001	0.0016	0.0015	-0.0003	0.0005
(percentile)	(0.0008)	(0.0006)	(0.0012)	(0.0010)	(0.0007)	(0.0005)
Children 6-17y (%)	-0.0070	0.0004	-0.0117	-0.0077	0.0017	-0.0021
	(0.0057)	(0.0063)	(0.0092)	(0.0079)	(0.0063)	(0.0070)
Physicians per 1000s	-0.0066	-0.0013	0.0072	0.0222**	-0.0006	-0.0004
	(0.0119)	(0.0054)	(0.0136)	(0.0102)	(0.0072)	(0.0065)
State FE	yes	yes	yes	yes	yes	yes
Cohort × Time FE	yes	yes	yes	yes	yes	yes
Mean DV	0.068	0.069	0.318	0.289	0.115	0.077
Observations	1,667	1,845	2,168	2,424	1,739	1,851
R^2	0.058	0.032	0.045	0.046	0.043	0.053

Notes: Sample restricted to birth cohorts from 1987 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.46: First Stage OLS Estimates on Take-up Rate (Binary)

	Take-up in 2006						
	(1)	(2)	(3)	(4)	(5)	(6)	(7)
Treated=1	0.147***	0.135***	0.135***	0.134***	0.134***	0.134***	0.136***
	(0.026)	(0.024)	(0.022)	(0.022)	(0.022)	(0.021)	(0.021)
SES Index			-0.003***	-0.003***	-0.003***	-0.003***	-0.003***
(percentile)			(0.000)	(0.000)	(0.000)	(0.000)	(0.000)
Mun. Marginality Index				0.003***	0.003***	0.002**	0.002*
(percentile)				(0.001)	(0.001)	(0.001)	(0.001)
Locality Controls							
Physicians per 1000s					-0.023***	-0.031***	-0.028***
					(0.006)	(0.007)	(0.007)
Illiteracy Rate (%)						0.016***	0.014**
						(0.005)	(0.006)
Mean Schooling (years)						0.035***	0.053***
						(0.012)	(0.013)
Members per Household							0.085
							(0.054)
Female (%)							-0.024**
							(0.011)
State FE	no	yes	yes	yes	yes	yes	yes
Cohort × Time FE	yes	yes	yes	yes	yes	yes	yes
Mean DV	144.7	144.7	144.7	144.7	144.7	144.7	144.7
Observations	5,714	5,714	5,714	5,714	5,714	5,714	5,714
R^2	0.118	0.155	0.261	0.278	0.283	0.291	0.298

Notes: Sample restricted to children born between 1987–1989 from urban localities not treated by 2000. Standard errors clustered by locality. All regressions include sample weights. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$.
Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.47: First Stage OLS Estimates on Take-up Rate (Continuous)

	Take-up in 2006						
	(1)	(2)	(3)	(4)	(5)	(6)	(7)
Years Treated	0.035***	0.032***	0.032***	0.032***	0.032***	0.032***	0.033***
	(0.006)	(0.006)	(0.005)	(0.005)	(0.005)	(0.005)	(0.005)
SES Index			-0.003***	-0.003***	-0.003***	-0.003***	-0.003***
(percentile)			(0.000)	(0.000)	(0.000)	(0.000)	(0.000)
Mun. Marginality Index				0.003***	0.003***	0.002**	0.002*
(percentile)				(0.001)	(0.001)	(0.001)	(0.001)
Locality Controls							
Physicians per 1000s					-0.023***	-0.031***	-0.028***
					(0.006)	(0.008)	(0.007)
Illiteracy Rate (%)						0.016***	0.014**
						(0.005)	(0.006)
Mean Schooling (years)						0.035***	0.054***
						(0.012)	(0.013)
Members per Household							0.088*
							(0.053)
Female (%)							-0.024**
							(0.011)
State FE	no	yes	yes	yes	yes	yes	yes
Cohort $\times$ Time FE	yes	yes	yes	yes	yes	yes	yes
Mean DV	144.7	144.7	144.7	144.7	144.7	144.7	144.7
Observations	5,714	5,714	5,714	5,714	5,714	5,714	5,714
R^2	0.119	0.156	0.263	0.280	0.284	0.292	0.300

Notes: Sample restricted to children born between 1987–1989 from urban localities not treated by 2000. Standard errors clustered by locality. All regressions include sample weights. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$.
Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.48: First Stage OLS Estimates on Take-up Rate (Categories)

	Take-up in 2006						
	(1)	(2)	(3)	(4)	(5)	(6)	(7)
Years Treated=4	0.127***	0.115***	0.113***	0.114***	0.113***	0.112***	0.111***
	(0.029)	(0.027)	(0.025)	(0.024)	(0.024)	(0.023)	(0.023)
Years Treated=5	0.197***	0.183***	0.187***	0.184***	0.184***	0.187***	0.199***
	(0.048)	(0.046)	(0.042)	(0.040)	(0.039)	(0.039)	(0.038)
SES Index			-0.003***	-0.003***	-0.003***	-0.003***	-0.003***
(percentile)			(0.000)	(0.000)	(0.000)	(0.000)	(0.000)
Mun. Marginality Index				0.003***	0.003***	0.002**	0.002*
(percentile)				(0.001)	(0.001)	(0.001)	(0.001)
Locality Controls							
Physicians per 1000s					-0.023***	-0.031***	-0.028***
					(0.006)	(0.008)	(0.007)
Illiteracy Rate (%)						0.016***	0.014**
						(0.005)	(0.006)
Mean Schooling (years)						0.036***	0.055***
						(0.012)	(0.013)
Members per Household							0.093*
							(0.052)
Female (%)							-0.025**
							(0.011)
State FE	no	yes	yes	yes	yes	yes	yes
Cohort × Time FE	yes	yes	yes	yes	yes	yes	yes
Mean DV	144.7	144.7	144.7	144.7	144.7	144.7	144.7
Observations	5,714	5,714	5,714	5,714	5,714	5,714	5,714
R^2	0.120	0.156	0.264	0.280	0.285	0.293	0.302

Notes: Sample restricted to children born between 1987–1989 from urban localities not treated by 2000. Standard errors clustered by locality. All regressions include sample weights. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$.
Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.49: LATE Estimates per Year on Anthropometric Measures

	Height (cm)		Weight (kg)		BMI	
	(1) Boys	(2) Girls	(3) Boys	(4) Girls	(5) Boys	(6) Girls
$\widehat{Progresa} = 1$	1.939**	0.810	2.853**	3.162**	0.560	1.557***
	(0.855)	(0.755)	(1.316)	(1.485)	(0.446)	(0.564)
SES Index (percentile)	0.097***	0.054*	0.165***	0.213***	0.042**	0.088***
	(0.035)	(0.029)	(0.056)	(0.057)	(0.020)	(0.022)
(SES Index) ^{2}	-0.0002	-0.0002	-0.0009*	-0.0016***	-0.0003*	-0.0007***
	(0.0003)	(0.0002)	(0.0005)	(0.0005)	(0.0002)	(0.0002)
Locality Controls						
Marginality (percentile)	0.019	-0.045***	0.009	-0.023	0.002	0.003
	(0.018)	(0.014)	(0.027)	(0.027)	(0.009)	(0.010)
Children 6-17y (%)	-0.488***	-0.131	-0.336	-0.409	-0.047	-0.203**
	(0.154)	(0.151)	(0.208)	(0.286)	(0.074)	(0.103)
Physicians per 1000s	0.649*	-0.060	0.556	0.885**	0.075	0.455***
	(0.370)	(0.306)	(0.351)	(0.395)	(0.125)	(0.121)
State FE	yes	yes	yes	yes	yes	yes
Cohort × Time FE	yes	yes	yes	yes	yes	yes
Mean DV	144.1	145.3	41.7	42.8	19.8	20.0
Observations	2,702	2,960	2,726	2,929	2,697	2,907
R^2	0.765	0.492	0.554	0.360	0.215	0.211

Notes: Sample restricted to birth cohorts from 1987 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.

Table A.50: LATE Estimates per Year on BMI Categories

	Underweight		Overweight		Obesity	
	(1) Boys	(2) Girls	(3) Boys	(4) Girls	(5) Boys	(6) Girls
$\widehat{Progresa} = 1$	0.0377	-0.0075	0.0001	0.1168*	0.0452	0.0718***
	(0.0415)	(0.0369)	(0.0588)	(0.0661)	(0.0303)	(0.0274)
SES Index	-0.0010	-0.0011	0.0030	0.0065***	0.0029**	0.0041***
	(0.0019)	(0.0013)	(0.0026)	(0.0023)	(0.0013)	(0.0011)
(SES Index)^2	0.00002	0.00001	-0.00003	-0.00005***	-0.00002	-0.00003***
	(0.00002)	(0.00001)	(0.00002)	(0.00002)	(0.00001)	(0.00001)
Locality Controls						
Marginality	-0.0005	-0.0001	0.0014	0.0004	-0.0005	-0.0002
(percentile)	(0.0008)	(0.0007)	(0.0012)	(0.0012)	(0.0008)	(0.0005)
Children 6-17y (%)	-0.0079	0.0003	-0.0106	-0.0133*	-0.0005	-0.0045
	(0.0051)	(0.0061)	(0.0088)	(0.0076)	(0.0060)	(0.0068)
Physicians per 1000s	0.0005	0.0028	0.0088	0.0317***	0.0043	0.0069
	(0.0114)	(0.0063)	(0.0168)	(0.0115)	(0.0084)	(0.0065)
State FE	yes	yes	yes	yes	yes	yes
Cohort × Time FE	yes	yes	yes	yes	yes	yes
Mean DV	0.066	0.067	0.317	0.285	0.115	0.072
Observations	1,792	2,022	2,339	2,647	1,872	2,026
R^2	0.053	0.030	0.034	0.039	0.038	0.047

Notes: Sample restricted to birth cohorts from 1987 to 1989 from urban localities treated between 2001-2005. All regressions include sample weights; standard errors clustered by locality. Individual covariates are interacted with a missing-value indicator to control for attrition bias. *** $p < 0.01$, ** $p < 0.05$, * $p < 0.10$. Sources: ENSANUT, Progresa Administrative Records, CONAPO, INEGI, Ministry of Health.